

Patient Face Sheet

TRANSCEND HOSPICE 18 LLC

PATIENT INFORMATION	
Patient Name: JERILYN HENRY	MR #: 97137
Admission Date: 11/26/2025	Admission #: 1 Current Status: Admitted (Hospice)
Primary Diagnosis (ICD10): I50.22 (Chronic systolic (congestive) heart failure)	
Related Conditions (ICD10): G31.1 (Senile degeneration of brain, not elsewhere classified), I48.91 (Unspecified atrial fibrillation), L03.90 (Cellulitis, unspecified), N39.46 (Mixed incontinence), F03.90 (Unsp dementia, unsp severity, without beh/psych/mood/anx), R53.1 (Weakness), Z91.81 (History of falling), R60.9 (Edema, unspecified), R19.7 (Diarrhea, unspecified), R06.03 (Acute respiratory distress)	
Unrelated Conditions (ICD10): Z85.831 (Personal history of malignant neoplasm of soft tissue)	
Level of Care: Routine Home Care	Disaster Code: Disaster Code 2
General Information:	
Birth Date: 1/18/1942	SSN: 321-34-4786
Race: White	Gender: Female
Ethnicity: Not of Hispanic, Latino/a, or Spanish origin	
Marital Status: Married	Is DNR?: Yes
Other Information:	Primary Language: English
Allergies: KNDA, Latex Natural Rubber	Religion: Baptist
Funeral Home: Address:	
Preferred Pharmacy Name: Ascript Rx Address: 17051 N Dallas Pkwy #110, Addison, TX 75001 Phone: (214) 643-6287 Fax: (888) 816-9765	
Patient Contact Information:	
Referral Information:	
Referral Source:	Called In By: Tiffani Hayes
LOCATION INFORMATION	
Current Location:	
Location Type: Mustang Creek Estates	
Institution Name (facility patients):	
Address: 720 Creek Valley Ct House 6 Room 8, Allen TX 75002-5294	
CAREGIVER INFORMATION	
Primary Caregiver	
Caregiver Name: Noel Starkweather	Relationship to Patient: Daughter
Address:	
Mobile: (503) 268-2460	
Secondary Caregiver	
Caregiver Name: Maria Luttrell	Relationship to Patient: Child
Address:	
Mobile: (205) 641-0570	
PHYSICIAN INFORMATION	
Attending Physician Name:	NPI #:
Address:	
Hospice Physician Name: RESTREPO, DIEGO	NPI #: 1063664076
FINANCIAL INFORMATION	
Payer Name: Palmetto GBA	
Coverage ID: 4E46RK5CU88 Coverage Type: Medicare Part A	
Service(s): Hospice (Primary)	

Medication List

HENRY, JERILYN (MR# 97137)
Allergies: KNDA, Latex Natural Rubber

Generated On: 12/22/2025 10:22 PM

Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
Regular Medications:									
11/26/2025		24 HR metoprolol succinate 100 MG Extended Release Oral Capsule	100 mg	1 tab Extended Release Oral Capsule	Oral Pill	am	HTN	Yes	Active
11/26/2025		acetaminophen 500 MG Oral Tablet [Tylenol]	500 mg	1 tab Oral Tablet	po	QID PRN	mild pain	Yes	Active
11/26/2025		apixaban 2.5 MG Oral Tablet [Eliquis]	2.5 mg	1 tab Oral Tablet	Oral Pill	bid	blood thinner	No	Active
11/26/2025		aspirin 81 MG Chewable Tablet	81 mg	1 tab Chewable Tablet	po	1 tab Daily	analgesics	Yes	Active
11/26/2025		donepezil hydrochloride 5 MG Oral Tablet [Aricept]	5 mg	1 tab Oral Tablet	Oral Pill	HS	Behaviors	Yes	Active
11/26/2025		lisinopril 10 MG Oral Tablet	10 mg	1 tab Oral Tablet	Oral Pill	daily	HTN	Yes	Active
11/26/2025		lovastatin 10 MG Oral Tablet	10 mg	1 tab Oral Tablet	Oral Pill	HS	HLD	No	Active
11/26/2025		sertraline 100 MG Oral Tablet	100 mg	1 tab Oral Tablet	Oral Pill	am	Depression	Yes	Active
11/26/2025		ubidecarenone 100 MG Oral Capsule [Q-Sorb Co Q-10]	100 mg	1 tab Oral Capsule	Oral Pill	daily AM	HTN	No	Active
12/4/2025		divalproex sodium 125 MG Delayed Release Oral Tablet [Depakote]	125 mg	1 cap Delayed Release Oral Tablet	Oral Pill	BID	behaviors	Yes	Active
11/26/2025	12/4/2025	quetiapine 25 MG Oral Tablet [Seroquel]	25 mg	1 tab Oral Tablet	po	BID	antipsychotic	Yes	Discontinued
Comfort Kit Medications:									
11/26/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
11/26/2025		Atropine sulfate oral Soluntion	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
11/26/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
11/26/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
11/26/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
11/26/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
11/26/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
11/26/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Figure: 25 TAC §157.25 (h)(2)



OUT-OF-HOSPITAL DO-NOT-RESUSCITATE (OOH-DNR) ORDER

TEXAS DEPARTMENT OF STATE HEALTH SERVICES

This document becomes effective immediately on the date of execution for health care professionals acting in out-of-hospital settings. It remains in effect until the person is pronounced dead by authorized medical or legal authority or the document is revoked. Comfort care will be given as needed.

Print Form

Person's full legal name

Jeryllyn Henry

Date of birth 1-18-1942

☒ Male
☐ Female

A. Declaration of the adult person: I am competent and at least 18 years of age. I direct that none of the following resuscitation measures be initiated or continued for me: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Person's signature

Noel Starkweather - POA

Date

11-25-2024

Printed name

Noel Starkweather

B. Declaration by legal guardian, agent or proxy on behalf of the adult person who is incompetent or otherwise incapable of communication: I am the: ☐ legal guardian; ☒ agent in a Medical Power of Attorney; OR ☐ proxy in a directive to physicians of the above-noted person who is incompetent or otherwise mentally or physically incapable of communication.

Based upon the known desires of the person, or a determination of the best interest of the person, I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Signature

Noel Starkweather

Date

11-25-2024

Printed name

Noel Starkweather

C. Declaration by a qualified relative of the adult person who is incompetent or otherwise incapable of communication: I am the above-noted person's: ☐ spouse, ☐ adult child, ☐ parent, OR ☐ nearest living relative, and I am qualified to make this treatment decision under Health and Safety Code §166.088.

To my knowledge the adult person is incompetent or otherwise mentally or physically incapable of communication and is without a legal guardian, agent or proxy. Based upon the known desires of the person or a determination of the best interests of the person, I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Signature

Date

Printed name

D. Declaration by physician based on directive to physicians by a person now incompetent or nonwritten communication to the physician by a competent person: I am the above-noted person's attending physician and have:

☐ seen evidence of his/her previously issued directive to physicians by the adult, now incompetent; OR

☐ observed his/her issuance before two witnesses of an OOH-DNR in a nonwritten manner.

I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Attending physician's signature

Date

Printed name

Lic#

E. Declaration on behalf of the minor person: I am the minor's: ☐ parent; ☐ legal guardian; OR ☐ managing conservator.

A physician has diagnosed the minor as suffering from a terminal or irreversible condition. I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Signature

Date

Printed name

TWO WITNESSES: (See qualifications on backside.) We have witnessed the above-noted competent adult person or authorized declarant making his/her signature above and, if applicable, the above-noted adult person making an OOH-DNR by nonwritten communication to the attending physician.

Witness 1 signature

Olivia L. Henry-Dur

Date

11/25/24

Printed name

Olivia L. Henry-Dur

Witness 2 signature

I. Attagumari

Date

11/25/24

Printed name

Ibrahim B Attagumari

Notary in the State of Texas and County of

The above noted person personally appeared before me and signed the above noted declaration on this date:

Signature & seal:

Notary's printed name:

Notary Seal

[Note: Notary cannot acknowledge the witnessing of the person making an OOH-DNR order in a nonwritten manner]

PHYSICIAN'S STATEMENT: I am the attending physician of the above-noted person and have noted the existence of this order in the person's medical records. I direct health care professionals acting in out-of-hospital settings, including a hospital emergency department, not to initiate or continue for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Physician's signature

Printed name

Huan Nguyen

Date

License #

8/4/25

124606

F. Directive by two physicians on behalf of the adult, who is incompetent or unable to communicate and without guardian, agent, proxy or relative: The person's specific wishes are unknown, but resuscitation measures are, in reasonable medical judgment, considered ineffective or are otherwise not in the best interests of the person. I direct health care professionals acting in out-of-hospital settings, including a hospital emergency department, not to initiate or continue for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Attending physician's signature

Huan Nguyen

Date

8/4/25

Printed name

Huan Nguyen

Lic#

124606

Signature of second physician

Date

Printed name

Lic#

Physician's electronic or digital signature must meet criteria listed in Health and Safety Code §166.082(c).

All persons who have signed above must sign below, acknowledging that this document has been properly completed.

Person's signature

Guardian/Agent/Proxy/Relative signature

Attending physician's signature

Second physician's signature

Witness 1 signature

Witness 2 signature

This document or a copy thereof must remain in the person's medical records.

Notary's signature

Patient: JERILYN HENRY
Medical Record: #97137

INITIAL COMPREHENSIVE ASSESSMENT (WITH HOPE)

COMPREHENSIVE ASSESSMENT

Date/Time In: 11/26/2025 9:45:00 AM
Date/Time Out: 11/26/2025 11:00:00 AM
Modality: In-person visit
Describe onset of illness, progression of symptoms, functional status, response to treatments, etc. including any recent hospitalizations, ER visits, weight loss and infections: Patient Name: Jerilyn Henry
Age/Sex/Race: 83-year-old Caucasian female
Date of Admission: 11/26/2025
Level of Care: Routine Home Care
Primary Diagnosis: I50.22 - Chronic systolic (congestive) heart failure
Related Comorbidities: G31.1 - Senile degeneration of brain, not elsewhere classified, I48.91 - Unspecified atrial fibrillation, L03.90 - Cellulitis, unspecified, N39.46 - Mixed incontinence, F03.90 - Unsp dementia, unsp severity, without beh/psych/mood/anx, R53.1 - Weakness, Z91.81 - History of falling, R60.9 - Edema, unspecified, R19.7 - Diarrhea, unspecified, R06.03 - Acute respiratory distress
Unrelated Conditions (if applicable): Z85.831 - Personal history of malignant neoplasm of soft tissue
Certifying Physician: Dr. Diego Restrepo
POA/Decision Maker: Step Daughter – Noelle Henry/ David Henry Spouse
Advance Directives: DNR;
Allergies: NKDA, Latex Natural Rubber
Current Scales: PPS: 50% KPS: 50% Mid-Arm Circumference (LMAC): 22FAST (if dementia): 6E NYHA Classification: Class IV (SOB at rest)
Subjective:
Past Medical History / DeclineThe patient has a significant cardiac history, including heart surgery with two stents in 2012 and a history of atrial fibrillation, although she was in a regular rhythm at today's visit. She carries a diagnosis of dementia, which has notably worsened over the last two weeks, with increased confusion, withdrawal, reduced ambulation, and poor safety awareness. She is a high fall risk and requires redirection and reminders for most tasks. Patient has a prior history of cellulitis to the leg requiring hospitalization and surgery in 2022 following a fall. She has a history of facial skin cancer and has required treatment in the past. She reports dysuria, periodic diarrhea, and decreased appetite with fatigue and generalized weakness. She reports drinking wine 2–3 times a year. Patients and family verbalize understanding of hospice philosophy and desire comfort-focused care.

ObjectiveGeneral: Cachectic, Pt SOB at rest SAT 89%on RA she refuses Oxygen. Lungs are clear, diminished throughout.
Neuro: A&O x2 but fatigued and slow to respond. Has a blank stare hoh.
Respiratory: Respirations 17 /min at rest; diminished breath sounds mid to lower lobes throughout; O2 saturation 88-89% on RA, She refuses Oxygen at this time.
Cardiac: HR 71, regular; BP 152/87 Pt with 2 stents placed in 2012, she has Afib
GI: Soft, non-tender; bowel sounds present; reports decreased appetite (50-75% of 2 meals). She may skin breakfast drinking coffee only
GU: Incontinent of urine just started on pull ups, toileting assistance is needed.
Skin: Dry, fragile, no open areas. Scab to left ankle noted d/t previous fall per cg 10 days ago
Extremities: Weak grip, generalized muscle wasting; NO lower-extremity edema.
Mobility: Pt ambulates with FWW high fall risk, weak needs assistance for safety.
Weight Change: unknown last weight per documentation was noted to be 114 lbs a year ago with Bmi of 21.56.
Functional Status
Current PPS and KPS are both 50%. She ambulates short distances with a four-wheel walker, but mobility has declined, and she spends increasing time in her room alone. She becomes easily confused, responds "huh?" frequently, and demonstrates poor short-term memory recall. She requires reminders to change clothes and may wear the same clothing for days. She resists showers and prefers "bird baths"; however, she is scheduled for three showers weekly with aide assistance. She is increasingly lethargic and fatigued, stating she "doesn't sleep," although observation indicates she naps frequently, approximately 10 hours cumulatively per day. Nutrition / IntakeShe eats 1–2 meals per day, consuming 50–75% of offered meals. She eats smaller portions at breakfast. She drinks a significant amount of coffee and reports occasional diarrhea. No edema noted. Respiratory / CardiovascularDuring this assessment, oxygen saturation was 88–89% on room air. She is refusing supplemental oxygen at this time. Lung sounds are shallow and clear/diminished throughout. Heart rhythm regular on assessment. GI / GUAbdomen is soft and non-tender. She is unsure of her last bowel movement but estimates "two days ago." Dysuria reported. No bladder distention noted. Psychosocial / BehavioralPatient is a loner, withdrawn, and prefers to stay to herself. She demonstrates increased anxiety and cognitive decline. She is forgetful and often responds to questions with "huh?" before attempting to answer. She takes quetiapine twice daily for behaviors and mood stabilization. Her husband is currently hospitalized for a hip fracture, which may be contributing to emotional withdrawal and decline. SpiritualShe identifies as Baptist and has Bibles placed throughout her room. She finds comfort in her faith. SafetyHigh fall riskPoor safety awarenessPoor short-term memoryIncreased confusionDeclining mobilityRequires repeated cues and remindersAssessment SummaryThe patient appears to be in overall decline, with worsening dementia, increasing weakness, reduced oral intake, increased fatigue, poor hygiene, and decreased ability to perform ADLs independently. Her oxygen saturation remains low on room air, and she refuses supplemental oxygen. Cognitive decline is progressing, and functional status has decreased to PPS/KPS 50%. Family is aware of her decline, agrees

with the hospice plan of care, and is supportive at this time. Ongoing education has been provided regarding disease progression, fall risk, and safety needs. Plan Continue routine home care hospice services Monitor cognitive decline, nutrition, hydration, bowel habits Reinforce fall precautions and safety Encourage oxygen use as tolerated Provide aide support for hygiene and showers Provide emotional and spiritual support Continue monitoring for changes in condition, symptoms, or needs Clinical

Summary Statement (CMS Required) Based on clinical findings, performance scale decline, respiratory compromise, progressive weight loss, increased dependence with ADLs, and overall functional decline, the patient demonstrates a terminal trajectory consistent with end-stage COPD. It is the nurse's clinical judgment that the patient has a life expectancy of six months or less should the disease follow its normal course. Hospice care is appropriate and medically necessary.

CO-MORBIDITIES

What other diagnoses does patient have that may contribute to a decreased life expectancy when combined with the primary diagnosis?

☒ Ischemic Heart Disease

☒ Coronary Artery Disease

☒ Generalized Muscle Weakness

☒ CVA

☒ Dementia

GENERAL INFORMATION

Race:

Caucasian

Religion:

Baptist

Primary Language:

English

Does the patient or primary caregiver need an interpreter?

No

PSYCHOSOCIAL / SPIRITUAL / BEREAVEMENT ASSESSMENT

Patient/Responsible Party's Current Response to Diagnosis/Prognosis: (Check all that apply)

• Acceptance

☒ Patient

☒ Representative

• Support Systems

☒ Other family not living with patient

Specify: Spouse

• Spiritual Support

Needs spiritual support? Yes

• Psychosocial Support

Needs psychosocial support? Yes

• Patient/Family Goals

☒ Patient and family will receive physical, spiritual, and emotional support

☒ Effective grieving

☒ Safe and comfortable dying

PAIN/COMFORT ASSESSMENT

Has patient experienced pain the past 7 days? Yes

Pain Assessment Scale utilized: PAIN-AD

Acceptable level of pain: 0

Best: 0

Worst: 10

☒ Generalized pain/undefined location

Pain Frequency: Intermittent

Pain Duration: 1-4 hours

☒ Patient is unable to describe character of pain

Non-Verbal Signs of Pain

☒ Moaning/crying

☒ Grimacing

☒ Guarding

☒ Restlessness

What makes the pain better?

☒ Relaxation techniques

☒ Medication

What causes pain or makes it worse?

☒ Standing

What other symptoms are associated with the pain?

☒ Poor concentration

☒ Decreased socialization

Current treatments for pain:

☒ NSAIDS

Patient is on a scheduled opioid, currently taking a PRN opioid or has been instructed to begin taking an opioid: No

How much breakthrough pain medication has patient taken in the past 24 hours? 0

Is current pain regime effective? Yes

Patient/family pain management goals Specific pain score (on a scale of 0-10)

Pain score (on a scale of 0-10):: 0

Patient/caregiver educated on pain regime and side effects of pain medications: Yes, and verbalized understanding

Patient educated on non-medication treatments of pain? Yes

☒ Heat

☒ Cold

Non-medication treatments were initiated: Yes

☒ Heat

☒ Cold

HEAD TO TOE ASSESSMENT

Vital Signs

Type of temperature taken: Temporal:
Temporal temperature: 96.4
Respiratory rate (respirations per minute): 18
Respiratory pattern: Regular
SaO2: 89
Pulse Type: Apical
Heart rate: 71
Blood pressure type: Laying
Systolic: 152
Diastolic: 87
Height: 61
Weight: 114
Arm circumference taken: Left
Arm circumference: 22

Nutrition

☒ Diet Orders (specify):

☒ General (as tolerated)

Average percentage of meals consumed: 75

Patient Feeding: Patient able to feed self independently

Comments: Pt will skip meals when she is upset she will only drink Coffee

Neurological Status**Orientation**

☒ Self

☒ Location

Behavior

☒ Calm

☒ Withdrawn

☒ Confused

☒ Forgetful

☒ Sad

Energy

☒ Fatigue/Weakness

Number of hours spent sleeping in a day: 10

Responsiveness

Responsiveness: Verbal stimuli

Intellect

Intellect: Short term memory loss

Speech

☒ Clear

Neuromusculoskeletal Status

☒ Shuffling gait/uncoordinated gait

Muscle Tone

☒ Muscle wasting

Functional Assessment

Functional Assessment Score: 6E

Gastrointestinal Status

Last BM Today

Bowel Sounds positive in all 4 Quadrants

☒ Normal

☒ RUQ

☒ RLQ

☒ LUQ

☒ LLQ

☒ Diarrhea

☒ Incontinence

Genitourinary Status

☒ Urinary incontinence

Comments: Pt is now incontinent of B/B

Integumentary Status

☒ Skin intact

Pressure Ulcer Risk Assessment

Sensory perception: ability to respond meaningfully to pressure-related discomfort:	1
Moisture: degree to which skin is exposed to moisture:	1
Activity: degree of physical activity:	1
Mobility: ability to change and control body position:	2
Nutrition: usual food intake pattern:	2
Friction and shear:	2
Total score:	9

CARDIOPULMONARY ASSESSMENT

Was patient screened for shortness of breath 1. Yes

Date of first screening for shortness of breath 11/26/2025

Did screening indicate the patient had shortness of breath 1. Yes

Was treatment for shortness of breath initiated 1. No, patient declined treatment

☒ Dyspnea

☒ At rest

☒ Decreased

☒ RML

☒ LLL

☒ LUL

☒ Cardiac Rhythm

☒ RRR

NY Heart Association Scale:

Class IV (Severe)

Comments:

Pt had heart Surgery 2012 2 stents were placed

FUNCTIONAL STATUS

Walking/Locomotion	Walker
Bed Mobility;	Independent
Transfers:	Assist of 1
Dressing:	Needs Assistance
Eating:	Needs Assistance
Toilet use:	Needs Assistance
Personal hygiene:	Needs Assistance
Bathing	Dependent
Palliative performance scale (PPS):	60% - Unable to do House Work
Comments:	Pt is declining she is a high fall risk and tends to be weak. She has had hx of previous falls.

Fall Risk Assessment (NHPCO Guidelines)

- History of a fall within the last 6 months
 - ☒ 2: 1 or 2 falls
 - Mobility
 - ☒ 2: Has a gait and/or balance problem
 - Elimination
 - ☒ 2: Intermittent incontinence
 - Mental Status
 - ☒ 2: Intermittent confusion
 - ☒ 1: Sensory impairment (hearing or vision)
 - Use of medication
 - ☒ 1: Psychotropic
 - Medical conditions which could impair the ability to ambulate
 - ☒ 1: Dementia
 - ☒ 1: Anti-hypertensives
- Total:

12
- ☒ 10 or more points = High risk

HOME SAFETY ASSESSMENT

☒ Patient lives in a facility that is deemed safe by the State

Comments: pt currently resides in ALF Mustng creek Allen

ASSESSMENT OF PATIENT/CAREGIVER TO SAFELY ADMINISTER MEDICATIONS

- ☒ N/A (Medication administered by facility staff)
- ☒ Medication may be safely administered by:
 - ☒ Caregiver
 - ☒ Nurse
- ☒ Patient/ Caregiver/POA educated on medication regime and side effects
- ☒ Patient/Caregiver/POA verbalized understanding of Medication teaching provided

HOPE Admission Assessment

- Based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less? 0. No
- Living Arrangements 3. Congregate home (alf, residential home care)
- Availability of Assistance 1. Around-the-clock (24 hours a day with few exceptions)
- Interpreter requested 0. No

Preferences

- Was patient/responsible party asked about CPR 1. Yes
 - Date asked about CPR 11/26/2025
- Was patient/responsible party asked about treatements other than CPR 1. Yes
 - Date asked about treatment other than CPR 11/26/2025
- Was patient/responsible party asked about hospitalization 1. Yes
 - Date asked about hospitalization 11/26/2025
- Was patient/responsible party asked about spiritual/existential concerns 1. Yes
 - Date asked about spiritual/existential concerns 11/26/2025

Active Diagnoses

Principal Diagnosis 06. Cardiovascular (excluding heart failure)

Comorbidities and Co-existing Conditions

- ☒ Heart Failure (e.g., congestive heart failure (CHF) and pulmonary edema)
- ☒ Sepsis
- ☒ Dementia (including Alzheimer's disease)

Pain

- Was patient screened for pain 1. Yes
 - Date of first screening for pain 11/26/2025
 - Patient's pain severity was 1. Mild
 - Type of standardized pain tool used 1. Numeric
- Is pain an active problem for the patient? 0. No

Symptom Impact

- Was a symptom impact screening completed? 1. Yes
 - Date of symptom impact screening 11/26/2025
 - Pain 1. Slight
 - Shortness of breath 1. Slight

Anxiety	2. Moderate
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	1. Slight
Constipation	1. Slight
Agitation	1. Slight

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No
Was PRN opioid initiated or continued? 0. No
Was bowel regimen initiated or continued? 2. Yes
Date bowel regimen initiated or continued 11/26/2025

TEACHING/EDUCATION PROVIDED

Education Provided To:

- ☒ Patient
- ☒ Caregiver

Comments: Spoke to Noell Pt Step Daughter pertaining to Pt admission, and condition

Topics

- ☒ Palliative care, hospice philosophy, hospice benefit
- ☒ Scope of services, role of team members
- ☒ Call us First Program
- ☒ Diagnosis
- ☒ Prognosis
- ☒ Disease process
- ☒ Safe & effective use of equipment & supplies
- ☒ Dying process, signs & symptoms of approaching death
- ☒ Covered medications, supplies, equipment
- ☒ How to re-order medications and supplies
- ☒ Medication usage, side effects, drug interactions
- ☒ Basic home safety, emergency preparedness
 - ☒ Provided written personalized emergency preparedness plan
 - ☒ Resides in a facility. Follow facility plan
- ☒ Infection control, disposal of hazardous waste, universal precautions
- ☒ Skin care, wound care, dressing changes
- ☒ Energy Conservation techniques
- ☒ Plan of care established with primary care giver
- ☒ Symptom management
- ☒ Catheter care

- ☒ Bowel regimen
- ☒ Pain management
- ☒ What to do at the time of death
- ☒ Anticipatory grief, grief, bereavement
- ☒ Funeral planning
- ☒ Advance directives
- ☒ Community resources
- ☒ Signs & symptoms of infection
- ☒ Discharge planning

Comments: Pt is currently a DNR PEr Daughter Paperwork was uploaded

COMMUNICATION WITH OTHER TEAM MEMBERS

- ☒ Attending Physician
- ☒ Hospice Medical Director
- ☒ Clinical Director/PCC
- ☒ Assigned RN Case Manager
- ☒ Social Worker
- ☒ Volunteer Coordinator
- ☒ Bereavement Coordinator
- ☒ Spiritual Counselor

Comments: Pt with significant decline over the past 2 weeks she is declining Poor STM loss and confusion noted, Pt has ability to become agitated easily. She Has had major Forgetfulness she remains a high fall risk and has a hx of cellulitis and sepsis. Previous hx of stent placement in 2012. Pt was noted to be SOB at rest.

Date: 11/26/2025

Electronically signed by: JOANNIE ESTES RN

Patient: JERILYN HENRY
Medical Record: #97137

SPIRITUAL ASSESSMENT

SPIRITUAL ASSESSMENT

Date/Time in: 11/26/2025 3:30:00 PM
Date/Time out: 11/26/2025 4:30:00 PM
Location/address: Mustang Creek Estates
RN case manager: Joanie Estes
Name/relationship of person providing information: Staff at Mustang Creek Estates and Patient

Response to Terminal Prognosis

• Acceptance

- ☒ Patient
- ☒ Representative

• Bargaining

- ☒ Patient

Pain Assessment

Pain rating: 0
Pain scale used: Verbal Scale

Religious Community

• Affiliation with a religious community

- ☒ Patient
- ☒ Representative

• Identifies with a faith tradition

- ☒ Patient

Specify: Christian- Baptist

Indicators of Spiritual Strengths

• Awareness of/sustained by the transcendent

- ☒ Patient
- ☒ Representative

• Has a sense of meaning, purpose, belonging

- ☒ Patient
- ☒ Representative

• Spiritually at peace

- ☒ Patient
- ☒ Representative

- *Trusting, hopeful*

☒ Patient☒ Representative

- *Belief in life after death/eternal life*

☒ Patient☒ Representative

- *Able to receive love/support from others*

☒ Patient☒ Representative

- *Able to love and forgive others*

☒ Patient☒ Representative

- *Ability to participate in laughter and celebration*

☒ Patient☒ Representative

- *Practices prayer, worship, and/or meditation*

☒ Patient☒ Representative

- *Ability to love and forgive self*

☒ Patient

- *Finds strength in spiritual rituals and/or symbols*

☒ Patient☒ Representative

- *Finds strength in community/kinship with others*

☒ Patient

- *Finds strength in sacred passages*

☒ Patient☒ Representative

- *Believes in the benevolence of a higher power*

☒ Patient☒ Representative

- *Sense of gratefulness/thanksgiving*

☒ Patient☒ Representative

- *Sense of humor*

☒ Patient

• *Ability to integrate faith and life experiences*

☒ Patient

☒ Representative

• *Ability to make commitments*

☒ Patient

☒ Representative

Comments related to spiritual strengths: Pt is a woman of very strong Christian beliefs and has multiple Bibles, inspirational books and wants to be an influence to others

Indicators of Spiritual Concerns/Spiritual Distress

• *Separation from faith community*

☒ Patient

• *Questioning the meaning of suffering, illness, death*

☒ Patient

Teaching and other interventions provided during visit (include patient/family's response to teaching/interventions):	Chaplain provided listening, spiritual care and support and discussion of pt beliefs. Chaplain brought meaning and spiritual resources to help pt cope. Pt family is supportive with both spouse and step daughter visiting pt frequently
--	---

Communication and Comments

Communication with other team members (include name, title, date, time, and topic(s) discussed):	Chaplain talked with RN case manager Jocelyn and pt step daughter and family
Comments related to Plan of Care and overall impressions:	Chaplain will plan to visit the pt on a 1x/month basis and PRN and provide spiritual guidance and support
Plan for next visit:	Build trust and encouruagmeent

Date: 11/26/2025

Electronically signed by: CLARK MAYS

Patient: JERILYN HENRY
Medical Record: #97137

Bereavement Risk Assessment

Date/Time In	11/26/2025 7:00:00 PM	
Date/Time Out	11/26/2025 7:15:00 PM	
Risk Assessment Grid		
Caregiver	Noel Starkweather (primary - Daughter)	Maria Luttrell (secondary - Child)
1: Possible alcohol/substance abuse	☐	☐
1: Neglect of appearance	☐	☐
1: History of mental or emotional health concerns	☐	☐
1: History of marked dependence on patient	☐	☐
1: History of suicide ideation or intent	☐	☐
1: Age less than 18	☐	☐
1: Absence of meaningful spiritual support	☐	☐
1: Death means loss of constant companion or emotional support	☐	☐
1: Death will result in loss of financial provisions/financial support	☐	☐
1: Death means change or loss of role or function	☐	☐
1: Unable to share feelings	☒	☐
1: Difficulty making decisions	☐	☐
1: Ambivalent relationship with the patient	☐	☐
1: Death means loss of meaning/purpose in life	☒	☒
1: Communication difficulties	☐	☐
1: Cultural issues present	☐	☐
1: Inflexible thinking	☐	☐
1: Dependent family members/children living in patient's home	☐	☐
1: Unfinished business with patient	☐	☒
1: Sense of hopelessness	☒	☒
1: Signs of spiritual distress	☐	☐
1: Estranged or isolated from support system	☐	☐
1: Inadequate or negative coping skills	☐	☐
1: Concurrent life crisis	☐	☐
1: Multiple or recent losses	☐	☐
1: Unprepared for death/strong denial prior to death	☐	☐
1: Difficulty coping with past losses	☐	☐
1: Unresolved grief from prior losses	☐	☐
1: Disenfranchised grief	☐	☐
1: History of family violence	☐	☐
1: Family or marital conflict	☐	☐
Risk Total:	3	3
Comments		

Electronically signed by: CLARK MAYS

Patient: JERILYN HENRY

Medical Record: #97137

HOSPICE PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Initial Certification of Terminal Illness

Start of Care Date: 11/26/2025
Benefit Period: First 90 Days
Certification Period - From: 11/26/2025
Certification Period - To: 2/23/2026
Primary Diagnosis: Chronic systolic (congestive) heart failure
ICD-10 Code: I50.22

Written Certification: Based on my clinical judgment, I certify that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Physician Narrative: Explain the clinical findings that support a life expectancy of six months or less. If applicable, include an explanation of why the findings of the face-to-face encounter support a life expectancy of six months or less.
Note: This narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.

Narrative: Patient 83-year-old male with terminal diagnosis of chronic systolic congestive heart failure. Patient comorbidities of senile gentian of the brain cellulitis incontinence dementia edema acute respiratory distress. Patient mated to hospice with the following declines in the past 3 to 6 months. Patient presents with PPS of 50% left MAC and 22 cm and fast score of 6E. Patient with NYHA of 4. Patient presents with shortness of breath at rest with saturation 89% on room air. Patient also presents with elevated blood pressures and irregular heart rate which goes along with his diagnosis of atrial fibrillation. Patient with recent incontinence of urine. Patient presents for generalized muscle weakness and wasting patient ambulates with walker however needs assistance for safety. Patient has become easily confused and demonstrating poor short-term memory recall. Patient hospice appropriate like expectancy less than 6 months.

Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD

Date: 11/26/2025

Patient: JERILYN HENRY
Medical Record: #97137

VERBAL PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Verbal Physician Initial Certification of Terminal Illness

Start of Care Date:

11/26/2025

Benefit Period:

First 90 Days

Certification Period - From:

11/26/2025

Certification Period - To:

2/23/2026

Primary Diagnosis:

Chronic systolic (congestive) heart failure

ICD-10 Code:

I50.22

Hospice Physician Initial Certification of Terminal Illness

Verbal Certification:

On

11/26/2025

Dr.

DIEGO F RESTREPO MD

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date:

11/26/2025

Physician Signature / Date:

Attending Physician Initial Certification of Terminal Illness

Verbal Certification:

On

11/26/2025

Dr.

DIEGO F RESTREPO MD

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date:

11/26/2025

Physician Signature / Date:

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Page 1 of 1

Beneficiary: Henry Jerilyn
DOD:

Medicare ID: 4E46RK5CU88

Sex: Female

DOB: 01/18/1942

Beneficiary Information

Beneficiary Last Name:*

Henry

Beneficiary First Name:**

Jerilyn

Medicare ID: *

4E46RK5CU88

Beneficiary Birth Date:**

01/18/1942

Beneficiary Name Suffix:

Optional Fields for Requesting Historical Data Using Date Range

Date From:

Date To:

Eligibility

Part A Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

01/01/2007

Termination Date:

Part B Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

01/01/2007

Termination Date:

Part B Immunosuppressive Drug Eligibility

Effective Date:

Termination Date:

Inactive Periods

Effective Date:

Termination Date:

Reason:

Beneficiary Address

Address Line 1:

3628 Piedmont Dr

Address Line 2:

City:

Plano

State:

TX

Zip:

75075-6235

End Stage Renal Disease (ESRD)

Coverage Period Effective Date:

Coverage Period End Date:

Dialysis Start Date:

Dialysis End Date:

Transplant Effective Date:

Additional Information

MBI End Date

End Date:

Audiology Screening

HCP Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

92550	07/01/2023		257	20%
92552	07/01/2023		257	20%
92553	07/01/2023		257	20%
92555	07/01/2023		257	20%
92556	07/01/2023		257	20%
92557	07/01/2023		257	20%
92562	07/01/2023		257	20%
92563	07/01/2023		257	20%
92565	07/01/2023		257	20%
92567	07/01/2023		257	20%
92568	07/01/2023		257	20%
92570	07/01/2023		257	20%
92571	07/01/2023		257	20%
92572	07/01/2023		257	20%
92575	07/01/2023		257	20%
92576	07/01/2023		257	20%
92577	07/01/2023		257	20%
92579	07/01/2023		257	20%
92582	07/01/2023		257	20%
92583	07/01/2023		257	20%
92584	07/01/2023		257	20%
92587	07/01/2023	07/01/2023	257	20%
92588	07/01/2023	07/01/2023	257	20%
92601	07/01/2023		257	20%
92602	07/01/2023		257	20%

HCPCS CodeNext Professional DateNext Technical DateRemaining DeductibleCo-insurance

92603	07/01/2023	257	20%
92604	07/01/2023	257	20%
92620	07/01/2023	257	20%
92621	07/01/2023	257	20%
92622	07/01/2023	257	20%
92623	07/01/2023	257	20%
92625	07/01/2023	257	20%
92626	07/01/2023	257	20%
92627	07/01/2023	257	20%
92640	07/01/2023	257	20%
92651	07/01/2023	257	20%
92652	07/01/2023	257	20%
92653	07/01/2023	257	20%

MDPP Coverage

Start Date:

End Date:

MDPP Inactive Periods

Start Date:

End Date:

MDPP Deductible

Start Date:

End Date:

Deductible Amount:

MDPP Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

MDPP Usage Information

HCPCS CodeBilling NPIDate Of Service

Deductibles/Caps

Acupuncture

Eligibility begins:

01/21/2020

Technical Sessions Remaining

20

Eligibility begins:

01/21/2020
Professional Sessions Remaining
20

Part B Deductible

Start Date:
01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$257

Part B Remaining Deductible

Start Date:
01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$0

Co-insurance Details

Start Date:
01/01/2025
End Date:
12/31/2025
Co-insurance Amount:
20%

Blood Deductible

Calendar Year:
2025
Number Of Units Remaining:
3

Occupational Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Physical And Speech Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Pulmonary Rehabilitation Services

Calendar Year:

2025
Professional Sessions Remaining:
72
Technical Sessions Remaining:
72

Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Intensive Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Part B Free Services [List of STC Codes](#)

STC Codes:
42,5,67,80,AJ,CO
Value:
0%
Start Date:
01/01/2025
End Date:
12/31/2025

Mental Health Co-insurance (if different from the plan level Co-insurance) [List of STC Codes](#)

STC Codes:
Value:
Start Date:
End Date:

Preventive

COVID

COVID-19 Immunization

Immunization Date	HCPC Code	Rendering Medicare NPI
No data available		

Pneumococcal Vaccine (PPV)

Pneumococcal Vaccine

Date of Service HCPC Code Rendering Medicare NPI

No data available

Flu

Flu Vaccination

Vaccination Date HCPC Code Rendering Medicare NPI

No data available

Cognitive Assessment and Care Plan Services

Cognitive Assessment and Care Plan Services

Date of Service HCPC Code Rendering Medicare NPI

No data available

Smoking Cessation

Number of Sessions in Benefit Period:

8

Initial Session Date:

Benefit Period Sessions Remaining:

8

Preventive

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

76706	07/01/2007	07/01/2007	0	0%
76977	01/01/2007	01/01/2007	0	0%
77067	01/01/2007	01/01/2007	0	0%
77078	01/01/2007	01/01/2007	0	0%
77080	01/01/2007	01/01/2007	0	0%
77081	01/01/2007	01/01/2007	0	0%
80061	01/01/2007	01/01/2007	0	0%
81528		10/09/2014	0	0%
82270	01/01/2007	01/01/2007	0	0%
82465	01/01/2007	01/01/2007	0	0%
82947	01/01/2007	01/01/2007	0	0%
82950	01/01/2007	01/01/2007	0	0%
82951	01/01/2007	01/01/2007	0	0%
83036	01/01/2024	01/01/2024	0	0%
83718	01/01/2007	01/01/2007	0	0%
84478	01/01/2007	01/01/2007	0	0%
G0475		04/13/2015	0	0%
G0473	01/01/2015	01/01/2015	0	0%
G0118	01/01/2007	01/01/2007	257	20%
G0117	01/01/2007	01/01/2007	257	20%
G0105	01/01/2007	01/01/2007	0	0%
G0104	01/01/2007	01/01/2007	0	0%
G0101	01/01/2007	01/01/2007	0	0%
G0447	11/29/2011	11/29/2011	0	0%
G0446	11/08/2011	11/08/2011	0	0%
G0445	11/08/2011	11/08/2011	0	0%
G0444	10/14/2011	10/14/2011	0	0%
G0443	02/05/2018		0	0%
G0442	02/01/2019		0	0%

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance				
G0148	01/01/2007	01/01/2007	0	0%
G0147	01/01/2007	01/01/2007	0	0%
G0145	01/01/2007	01/01/2007	0	0%
G0144	01/01/2007	01/01/2007	0	0%
G0143	01/01/2007	01/01/2007	0	0%
G0439	01/01/2011		0	0%
G0438	01/01/2011		0	0%
P3000	01/01/2007	01/01/2007	0	0%
G0130	01/01/2007	01/01/2007	0	0%
G0328	01/01/2007	01/01/2007	0	0%
G0327		07/01/2021	0	0%
G0123	01/01/2007	01/01/2007	0	0%
G0121	01/01/2007	01/01/2007	0	0%
Q0091	01/01/2007	01/01/2007	0	0%

Hepatitis Screening

Date Of Service HCPCS Code Provider or Rendering NPI Deductible Co-insurance

Plan Coverage

Medicare Advantage

Plan Type:

Preferred Provider Organization(PPO)

Enrollment Date:

07/01/2024

Disenrollment Date:

Contract Number:

H3288

Contract Name:

AETNA HEALTH AND LIFE INSURANCE COMPANY

Plan Number:

002

Plan Name:

Aetna Medicare Value Plus

Address Line 1:

151 Farmington Ave.

Phone Number:

8006240756

Address Line 2:

City:

Hartford

State:

CT

Zip Code:

06156

Website

<https://www.aetnamedicare.com>

Bill Code :

C

VBID Model Hospice Benefit Component Links

[Hospice Benefit Component participating plans](#)

[Directions for submitting claims for plans supporting the Hospice Benefit Component of the Model](#)

Medicare Part D

Enrollment Date:

07/01/2024

Disenrollment Date:

Contract Number:

H3288

Contract Name:

AETNA HEALTH AND LIFE INSURANCE COMPANY

Plan Number:

002

Plan Name:

Aetna Medicare Value Plus

Address Line 1:

151 Farmington Ave.

Phone Number:

8006240756

Address Line 2:

City:

Hartford

State:

CT

Zip Code:

06156

Website

<https://www.aetnamedicare.com>

Drug Plan :

OT

Enrollment

Y

Medicare Secondary Payer

Medicare Secondary Payer

Effective Date:

Termination Date:

Insurer Name:

Policy Number:

Patient Relationship:

Group Number:

Type of Primary Insurance:

Diagnosis Codes:

Address Line 1:

Address Line 2:

City:

State:

Zip Code:

Source Code:

Maintenance Date:

Ongoing Responsibility For Medicals:

Hospice/HomeHealth

Home Health Care

Patient Status:

NOA Indicator:

HHEH Start Date:

HHEH End Date:

HHEH DOEBA Date:

HHEH DOLBA Date:

Provider Number:

Provider Number Type:

Contractor Number:

Contractor Name:

HH Certification Start Date(s):

HH Recertification Start Date(s):

Hospice

Hospice Episodes:						
Effective Date	Termination Date	Start Date (DOEBA)	End Date (DOLBA)	Hospice Days Used	Provider Number	Provider Number Type

Notices of Election (NOE):

Date	Election Receipt Date	Provider Number	Provider Number Type	Revocation Code	Election Revocation Date
------	-----------------------	-----------------	----------------------	-----------------	--------------------------

Part A Deductible

Start Date:
01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$1676

Part A Base Deductible Remaining

Start Date:
01/01/2025
End Date:
12/31/2025
Remaining Deductible:
\$1676

Deductible Remaining By Spell

DOEBA Date	DOLBA Date	Deductible Amt
No data to display		

Inpatient Spell Dates

Includes Hospital and Skilled Nursing Facility (SNF) Dates

No data to display

Inpatient Base Summary

Start Date	End Date	Full Days Allowed	Full Copayment Days Allowed	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full Inpatient Days	Full Inpatient Copay Days	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

SNF Base Summary

Start Date	End Date	SNF Full Days Allowed	SNF Full Copayment Days Allowed	SNF Copay Amount
01/01/2025	12/31/2025	20	80	\$209.5

SNF Days Remaining (May Also Include Spells)

DOEBA Date DOLBA Date Full SNF Days Full SNF Copay Days SNF Copay Amount

01/01/2025 12/31/2025 20 80 209.5

Lifetime Reserve Days

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Lifetime Days Allowed:

Lifetime Days Remaining :

Calendar Year:

Copayment Amount Per Day:

Lifetime Psychiatric Benefit Data

Lifetime Psychiatric Base Days:

190

Lifetime Psychiatric Remaining:

190

Part A Free Services

List of STC Codes

STC Codes:

42,45

Value:

0%

Start Date:

01/01/2025

End Date:

12/31/2025

Qualified Medicare Beneficiary (QMB) Program Eligibility

Beneficiaries who are enrolled in the Qualified Medicare Beneficiary (QMB) program are dually eligible for both Medicare and Medicaid. Beneficiaries who are enrolled in the QMB program are not liable for Medicare co-insurance or deductible payments. Note that QMB status may fluctuate for a minority of Beneficiaries. If eligibility results indicate the Beneficiary QMB enrollment has terminated, please verify the patient's QMB status through State online Medicaid eligibility systems or other documentation, including Medicaid Identification cards and documents issued by the State proving the patient qualifies for the QMB program.

Medicaid Enrollment

State:

Enrollment Date:

Termination Date:

Part A Deductible

Start Date:

End Date:

Deductible Amount:

Inpatient Base Summary

Start Date:

End Date:

Full Days Allowed:

Full Copay Days Allowed:

Copay Amount:

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date:

DOLBA Date:

Full Inpatient Days:

Full Inpatient Copay Days:

Copay Amount:

SNF Base Summary

Start Date:

End Date:

SNF Full Days Allowed:

SNF Full Copay Days:

Copay Amount:

SNF Days Remaining (May Also Include Spells)

DOEBA Date:

DOLBA Date:

Full SNF Days:

Full SNF Copay Days:

Copay Amount:

Lifetime Reserve

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Start Date:

End Date:

Copayment Amount:

Part B Deductible

Start Date:

End Date:

Deductible Amount:

Part B Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

Beneficiary: Henry Jerilyn
DOD:

Medicare ID: 4E46RK5CU88

Sex: Female

DOB: 01/18/1942

Beneficiary Information

Beneficiary Last Name:*

Beneficiary First Name:**

Medicare ID: *

Beneficiary Birth Date:**

Beneficiary Name Suffix:

Optional Fields for Requesting Historical Data Using Date Range

Date From:

Date To:

Eligibility

Part A Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

01/01/2007

Termination Date:

Part B Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

01/01/2007

Termination Date:

Part B Immunosuppressive Drug Eligibility

Effective Date:

Termination Date:

Inactive Periods

Effective Date:

Termination Date:

Reason:

Beneficiary Address

Address Line 1:

3628 Piedmont Dr

Address Line 2:

City:

Plano

State:

TX

Zip:

75075-6235

End Stage Renal Disease (ESRD)

Coverage Period Effective Date:

Coverage Period End Date:

Dialysis Start Date:

Dialysis End Date:

Transplant Effective Date:

Additional Information

MBI End Date

End Date:

Audiology Screening

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

92550	07/01/2023	257	20%
92552	07/01/2023	257	20%
92553	07/01/2023	257	20%
92555	07/01/2023	257	20%
92556	07/01/2023	257	20%
92557	07/01/2023	257	20%
92562	07/01/2023	257	20%
92563	07/01/2023	257	20%
92565	07/01/2023	257	20%
92567	07/01/2023	257	20%
92568	07/01/2023	257	20%
92570	07/01/2023	257	20%
92571	07/01/2023	257	20%
92572	07/01/2023	257	20%
92575	07/01/2023	257	20%
92576	07/01/2023	257	20%
92577	07/01/2023	257	20%
92579	07/01/2023	257	20%

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

92582	07/01/2023		257	20%
92583	07/01/2023		257	20%
92584	07/01/2023		257	20%
92587	07/01/2023	07/01/2023	257	20%
92588	07/01/2023	07/01/2023	257	20%
92601	07/01/2023		257	20%
92602	07/01/2023		257	20%
92603	07/01/2023		257	20%
92604	07/01/2023		257	20%
92620	07/01/2023		257	20%
92621	07/01/2023		257	20%
92622	07/01/2023		257	20%
92623	07/01/2023		257	20%
92625	07/01/2023		257	20%
92626	07/01/2023		257	20%
92627	07/01/2023		257	20%
92640	07/01/2023		257	20%
92651	07/01/2023		257	20%
92652	07/01/2023		257	20%
92653	07/01/2023		257	20%

MDPP Coverage

Start Date:

End Date:

MDPP Inactive Periods

Start Date:

End Date:

MDPP Deductible

Start Date:

End Date:

Deductible Amount:

MDPP Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

MDPP Usage Information**HCPCS Code Billing NPI Date Of Service**

Deductibles/Caps

Acupuncture

Eligibility begins:

01/21/2020

Technical Sessions Remaining

20

Eligibility begins:

01/21/2020

Professional Sessions Remaining

20

Part B Deductible

Start Date:

01/01/2025

End Date:

12/31/2025

Deductible Amount:

\$257

Part B Remaining Deductible

Start Date:

01/01/2025

End Date:

12/31/2025

Deductible Amount:

\$0

Co-insurance Details

Start Date:

01/01/2025

End Date:

12/31/2025

Co-insurance Amount:

20%

Blood Deductible

Calendar Year:

2025

Number Of Units Remaining:

3

Occupational Therapy Cap

Calendar Year:

2025

Amount Used:
\$0

Physical And Speech Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Pulmonary Rehabilitation Services

Calendar Year:
2025
Professional Sessions Remaining:
72
Technical Sessions Remaining:
72

Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Intensive Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Part B Free Services [List of STC Codes](#)

STC Codes:
42,5,67,80,AJ,CO
Value:
0%
Start Date:
01/01/2025
End Date:
12/31/2025

Mental Health Co-insurance (if different from the plan level Co-insurance) [List of STC Codes](#)

STC Codes:

Value:

Start Date:

End Date:

Preventive

COVID

COVID-19 Immunization

Immunization Date HCPC Code Rendering Medicare NPI

No data available

Pneumococcal Vaccine (PPV)

Pneumococcal Vaccine

Date of Service HCPC Code Rendering Medicare NPI

No data available

Flu

Flu Vaccination

Vaccination Date HCPC Code Rendering Medicare NPI

No data available

Cognitive Assessment and Care Plan Services

Cognitive Assessment and Care Plan Services

Date of Service HCPC Code Rendering Medicare NPI

No data available

Smoking Cessation

Number of Sessions in Benefit Period:

8

Initial Session Date:

Benefit Period Sessions Remaining:

8

Preventive

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

74263	01/01/2025	01/01/2025	0	0%
76706	07/01/2007	07/01/2007	0	0%
76977	01/01/2007	01/01/2007	0	0%
77067	01/01/2007	01/01/2007	0	0%
77078	01/01/2007	01/01/2007	0	0%
77080	01/01/2007	01/01/2007	0	0%
77081	01/01/2007	01/01/2007	0	0%
80061	01/01/2007	01/01/2007	0	0%
81528		10/09/2014	0	0%

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

82270	01/01/2007	01/01/2007	0	0%
82465	01/01/2007	01/01/2007	0	0%
82947	01/01/2007	01/01/2007	0	0%
82950	01/01/2007	01/01/2007	0	0%
82951	01/01/2007	01/01/2007	0	0%
83036	01/01/2024	01/01/2024	0	0%
83718	01/01/2007	01/01/2007	0	0%
84478	01/01/2007	01/01/2007	0	0%
G0473	01/01/2015	01/01/2015	0	0%
G0118	01/01/2007	01/01/2007	257	20%
G0117	01/01/2007	01/01/2007	257	20%
G0105	01/01/2007	01/01/2007	0	0%
G0104	01/01/2007	01/01/2007	0	0%
G0101	01/01/2007	01/01/2007	0	0%
G0447	11/29/2011	11/29/2011	0	0%
G0446	11/08/2011	11/08/2011	0	0%
G0445	11/08/2011	11/08/2011	0	0%
G0444	10/14/2011	10/14/2011	0	0%
G0443	02/05/2018		0	0%
G0442	02/01/2019		0	0%
G0148	01/01/2007	01/01/2007	0	0%
G0147	01/01/2007	01/01/2007	0	0%
G0145	01/01/2007	01/01/2007	0	0%
G0144	01/01/2007	01/01/2007	0	0%
G0143	01/01/2007	01/01/2007	0	0%
G0439	01/01/2011		0	0%
G0438	01/01/2011		0	0%
P3000	01/01/2007	01/01/2007	0	0%
0464U		10/09/2014	0	0%
G0130	01/01/2007	01/01/2007	0	0%
G0328	01/01/2007	01/01/2007	0	0%
G0327		07/01/2021	0	0%
G0123	01/01/2007	01/01/2007	0	0%
G0121	01/01/2007	01/01/2007	0	0%
Q0091	01/01/2007	01/01/2007	0	0%

Hepatitis Screening**Date Of Service HCPCS Code Provider or Rendering NPI Deductible Co-insurance****HIV/PrEP - Pre-Exposure Prophylaxis for HIV Prevention****Date of Service HCPCS Code Provider or Rendering NPI Deductible Co-insurance**

No data available

Plan Coverage**Medicare Advantage**

Plan Type:

Preferred Provider Organization(PPO)

Enrollment Date:

07/01/2024

Disenrollment Date:

Contract Number:

H3288

Contract Name:

AETNA HEALTH AND LIFE INSURANCE COMPANY

Plan Number:

002

Plan Name:

Aetna Medicare Value Plus

Address Line 1:

151 Farmington Ave.

Phone Number:

8006240756

Address Line 2:

City:

Hartford

State:

CT

Zip Code:

06156

Website

<https://www.aetnamedicare.com>

Bill Code :

C

VBID Model Hospice Benefit Component Links[Hospice Benefit Component participating plans](#)[Directions for submitting claims for plans supporting the Hospice Benefit Component of the Model](#)**Medicare Part D**

Enrollment Date:

07/01/2024

Disenrollment Date:

Contract Number:

H3288

Contract Name:

AETNA HEALTH AND LIFE INSURANCE COMPANY

Plan Number:

002

Plan Name:

Aetna Medicare Value Plus

Address Line 1:

151 Farmington Ave.

Phone Number:

8006240756

Address Line 2:

City:

Hartford

State:

CT

Zip Code:

06156

Website

<https://www.aetnamedicare.com>

Drug Plan :

OT

Enrollment

Y

Medicare Secondary Payer

Medicare Secondary Payer

Effective Date:

Termination Date:

Insurer Name:

Policy Number:

Patient Relationship:

Group Number:

Type of Primary Insurance:

Diagnosis Codes:

Address Line 1:

Address Line 2:

City:

State:

Zip Code:

Source Code:

Maintenance Date:

Ongoing Responsibility For Medicals:

Hospice/HomeHealth

Home Health Care

Patient Status:

NOA Indicator:

HHEH Start Date:

HHEH End Date:

HHEH DOEBA Date:

HHEH DOLBA Date:

Provider Number:

Provider Number Type:

Contractor Number:

Contractor Name:

HH Certification Start Date(s):

HH Recertification Start Date(s):

Hospice

Hospice Episodes:

Effective Date	Termination Date	Start Date (DOEBA)	End Date (DOLBA)	Hospice Days Used	Provider Number	Provider Number Type
----------------	------------------	--------------------	------------------	-------------------	-----------------	----------------------

Notices of Election (NOE):

Date	Election Receipt Date	Provider Number	Provider Number Type	Revocation Code	Election Revocation Date
------	-----------------------	-----------------	----------------------	-----------------	--------------------------

Part A Deductible

Start Date:

01/01/2025

End Date:

12/31/2025

Deductible Amount:

\$1676

Part A Base Deductible Remaining

Start Date:

01/01/2025

End Date:

12/31/2025

Remaining Deductible:

\$1676

Deductible Remaining By Spell

DOEBA Date DOLBA Date Deductible Amt

No data to display

Inpatient Spell Dates

Includes Hospital and Skilled Nursing Facility (SNF) Dates

No data to display

Inpatient Base Summary

Start Date	End Date	Full Days Allowed	Full Copayment Days Allowed	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full Inpatient Days	Full Inpatient Copay Days	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

SNF Base Summary

Start Date	End Date	SNF Full Days Allowed	SNF Full Copayment Days Allowed	SNF Copay Amount
01/01/2025	12/31/2025	20	80	\$209.5

SNF Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full SNF Days	Full SNF Copay Days	SNF Copay Amount
01/01/2025	12/31/2025	20	80	209.5

Lifetime Reserve Days

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Lifetime Days Allowed:

Lifetime Days Remaining :

Calendar Year:

Copayment Amount Per Day:

Lifetime Psychiatric Benefit Data

Lifetime Psychiatric Base Days:

190

Lifetime Psychiatric Remaining:

190

Part A Free Services List of STC Codes

STC Codes:

42,45

Value:

0%

Start Date:

01/01/2025

End Date:

12/31/2025

Qualified Medicare Beneficiary (QMB) Program Eligibility

Beneficiaries who are enrolled in the Qualified Medicare Beneficiary (QMB) program are dually eligible for both Medicare and Medicaid. Beneficiaries who are enrolled in the QMB program are not liable for Medicare co-insurance or deductible payments. Note that QMB status may fluctuate for a minority of Beneficiaries. If eligibility results indicate the Beneficiary QMB enrollment has terminated, please verify the patient's QMB status through State online Medicaid eligibility systems or other documentation, including Medicaid Identification cards and documents issued by the State proving the patient qualifies for the QMB program.

Medicaid Enrollment

State:

Enrollment Date:

Termination Date:

Part A Deductible

Start Date:

End Date:

Deductible Amount:

Inpatient Base Summary

Start Date:

End Date:

Full Days Allowed:

Full Copay Days Allowed:

Copay Amount:

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date:

DOLBA Date:

Full Inpatient Days:

Full Inpatient Copay Days:

Copay Amount:

SNF Base Summary

Start Date:

End Date:

SNF Full Days Allowed:

SNF Full Copay Days:

Copay Amount:

SNF Days Remaining (May Also Include Spells)

DOEBA Date:

DOLBA Date:

Full SNF Days:

Full SNF Copay Days:

Copay Amount:

Lifetime Reserve

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Start Date:

End Date:

Copolyment Amount:

Part B Deductible

Start Date:

End Date:

Deductible Amount:

Part B Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Jerilyn Henry, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% – Medicare Beneficiary Identifier (MBI): 4E46-RK5-CU88 Effective Date: 01/01/2007

☐ Bill Medicaid 100% – Medicaid Beneficiary Number: _____ Effective Date: _____

☐ Bill Primary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

☐ Bill Secondary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

Benefits are: _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST

☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety Information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D Information
- Home Hazardous Waste Disposal Information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

Beneficiary/Patient Signature

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:30 CST

Date

November 25, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

Daughter

Cognitive Decline due to Advanced DX

Relationship to Patient

Reason patient is unable to sign

E-SIGNED by Jomya Pollard
on 2025-11-25 16:54:35 CST

November 25, 2025

Hospice Agency Representative Signature

Date

Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Jerilyn Henry, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 11-26-25 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentraqio.com, phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - Provided by the designated Hospice.
 - Provided by another hospice under arrangements made by the designated Hospice.
 - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☐ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) 1063664076

Office Address: _____

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☐ N/A Patient is non-Medicare

☐ N/A I was transferred from _____ on _____.

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Homage Hospice _____ with the effective date of election to begin on 11-26-25.

I am electing the (1st, 2nd, 3rd, 4th, etc.) 1st _____ benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

Beneficiary/Patient Signature _____

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:54 CST

Date

November 25, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign) _____

Date

Daughter

Cognitive Decline Due to Advanced DX

Relationship to Patient _____

Reason patient is unable to sign _____

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:36 CST

November 25, 2025

Hospice Agency Representative Signature _____

Date

Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs

Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☐ I am not requesting the addendum. I have been made aware that I can request it at any time.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:36 CST

November 25, 2025

Signature of Beneficiary/Patient's Authorized Representative

Date

POA/ Daughter/ Cognitive Decline Due to Advanced DX
Representative's Relationship/Reason Patient Unable to Sign

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:38 CST

November 25, 2025

Signature of Witness/Hospice Representative

Date

The Hospice furnished the addendum on:

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Jerilyn Henry.

I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

- I. I understand the Hospice's obligations are as follows:
 - A. The Hospice staff will provide instructions to me as needed.
 - B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.
 - C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.
- II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.
- III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:38 CST

November 25, 2025

Caregiver Signature

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:40 CST

November 25, 2025

Hospice Signature

Date

No Primary Caregiver Agreement

I, Jerilyn Henry, do agree and will comply with the following:

- Understand that I have an illness which has been diagnosed as appropriate for hospice care.
- Understand the goal of hospice care is to relieve, control or minimize symptoms to maintain optimal quality of life in patients with a life limiting illness. Treatment and diagnostic procedures which would cause increased discomfort without increased quality of life will be avoided.
- At the present time I live alone with no primary caregiver.
- When the Hospice Team determines that, due to disease progression or changes in my condition, it is unsafe for me to remain in my home alone, I agree to the following change of my environment. This may include contacting or hiring someone to provide 24-hour care to me.

1. _____
2. _____
3. _____
4. _____

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:57 CST

Patient Signature

November 25, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:41 CST

Witness

November 25, 2025

Date

Patient Choice Statement
(To be completed for all new patients/guardians)

I, Jerilyn Henry (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Homage Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:42 CST

November 25, 2025

Signature of Patient/Guardian

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:43 CST

November 25, 2025

Signature of Hospice Representative

Date

Hospice Wound Photography Consent

I, Jerilyn Henry _____ hereby consent and so authorize the taking
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 11-26-25

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:45 CST

November 25, 2025

Patient Signature (or other authorized signature)

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:46 CST

November 25, 2025

Witness Signature

Date

Daughter

Relationship, if other than patient

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:47 CST

Beneficiary/Patient/Authorized Representative Signature

November 25, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:48 CST

Hospice Agency Representative Signature

November 25, 2025

Date



HOMAGE
—HOSPICE—

Homage Hospice
8204 Elmbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name Jerilyn Henry Birth Date 01/18/1942
Address 720 Creek Valley Ct
City Allen State TX Zip 75002
Home Phone _____ Business Ph _____ Cell _____

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

****Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.****

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof. "Medical Records" includes the following:
Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Hospice Tools in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:51 CST

November 25, 2025

Patient/Authorized Signature

Date

Medicare Secondary Payer Screening Form

Patient Name: Jerilyn Henry

SOC: 11-26-25

Date: 11-26-25

PART I

- Are you receiving Black Lung (BL) Benefits?
☒ No Go to 2.
☐ Yes BL is primary payer only for claims related to BL. Date benefits began: _____
- Are the services to be paid by a government research program?
☒ No Go to 3.
☐ Yes Government Research Program will pay primary benefits for these services.
- Has the Department of Veterans Affairs (DVA) authorized and agreed to pay for your care at this facility?
☒ No Go to 4.
☐ Yes DVA is primary for these services.
- Was the illness/injury due to a work-related accident/condition?
☒ No Go to Part II
☐ Yes Go to Insurance Information Form

PART II

- Was illness/injury due to a non-work-related accident?
☒ No Go to Part III
☐ Yes Go to Insurance Information Form

WC is primary payer only for related claims for work-related

PART III

- Are you entitled to Medicare based on:
☒ Age- Go to PART IV

Disability- Go to PART V

PART IV - AGE

- Are you or your spouse currently employed?
Patient ☒ No - Never worked No - Retired Date: _____
Spouse ☒ No - Never worked No - Retired Date: _____
If the patient answers "no" to 2, Medicare is primary unless answered "yes" to questions in part I or II.

End-Stage Renal Disease (ESRD- Go to PART VI.

- Do you have group health plan (GHP) coverage based on your own or a spouse's current employer who employs 20 or > employees?
Patient ☒ No Yes, GHP is primary Go to Insurance Information Form
Spouse ☒ No Yes, GHP is primary Go to Insurance Information Form
If the patient answers "no" to all, Medicare is primary unless answered "yes" to questions in part I or II.

PART V - DISABILITY

- Do you have group health plan (GHP) coverage based on your own, a spouse or family member's current employer who employs 100 or > employees?
Patient ☒ No Yes, GHP is primary Go to Insurance Information Form
Spouse ☒ No Yes, GHP is primary Go to Insurance Information Form
Family ☒ No Yes, GHP is primary Go to Insurance Information Form

PART VI - ESRD

- Do you have group health plan (GHP) coverage based on your own, a spouse or family member?
Patient ☒ No Yes, GHP is primary Go to Insurance Information Form
Spouse ☒ No Yes, GHP is primary Go to Insurance Information Form
Family ☒ No Yes, GHP is primary Go to Insurance Information Form
- Have you received a kidney transplant?
☒ No Date of Transplant: _____
☐ Yes
- Have you received maintenance dialysis treatments?
☒ No
☐ Yes Dialysis Start Date: _____
- Are you within the 30-month coordination period that starts MM/DD/CCYY? (The 30-month coordination period starts the first day of the month an individual is eligible for Medicare (even if not yet enrolled in Medicare) because of kidney failure (usually the fourth month of dialysis). If the individual is participating in a self-dialysis training program or has a kidney transplant during the 3-month waiting period, the 30-month coordination period starts with the first day of the month of dialysis or kidney transplant.)
☒ No Medicare is Primary
☐ Yes If app, Self-Dialysis Training Program Start Date: _____

- Are you entitled to Medicare on the basis of either ESRD and age or ESRD and disability?
☒ No
☐ Yes

- Was your initial entitlement to Medicare (including simultaneous or dual entitlement) based on ESRD?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.

- Does the working aged or disability MSP provision apply (i.e., is the GHP already primary based on age or disability entitlement)?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.

Hospice Signature: [Signature]

Date: 11-26-25

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Jerilyn Henry, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% – Medicare Beneficiary Identifier (MBI): 4E46-RK5-CU88 Effective Date: 01/01/2007

☐ Bill Medicaid 100% – Medicaid Beneficiary Number: _____ Effective Date: _____

☐ Bill Primary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

☐ Bill Secondary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

Benefits are: _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST

☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety Information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D Information
- Home Hazardous Waste Disposal Information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

Beneficiary/Patient Signature

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:30 CST

Date

November 25, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

Daughter

Cognitive Decline due to Advanced DX

Relationship to Patient

Reason patient is unable to sign

E-SIGNED by Jomya Pollard
on 2025-11-25 16:54:35 CST

November 25, 2025

Hospice Agency Representative Signature

Date

Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Jerilyn Henry, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 11-26-25 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentraqio.com, phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - Provided by the designated Hospice.
 - Provided by another hospice under arrangements made by the designated Hospice.
 - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☐ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) 1063664076

Office Address: _____

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☐ N/A Patient is non-Medicare

☐ N/A I was transferred from _____ on _____.

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Homage Hospice _____ with the effective date of election to begin on 11-26-25.

I am electing the (1st, 2nd, 3rd, 4th, etc.) 1st _____ benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

Beneficiary/Patient Signature _____

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:54 CST

Date

November 25, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign) _____

Date

Daughter

Cognitive Decline Due to Advanced DX

Relationship to Patient _____

Reason patient is unable to sign _____

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:36 CST

November 25, 2025

Hospice Agency Representative Signature _____

Date

Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs

Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☐ I am not requesting the addendum. I have been made aware that I can request it at any time.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:36 CST

Signature of Beneficiary/Patient's Authorized Representative

November 25, 2025

Date

POA/ Daughter/ Cognitive Decline Due to Advanced DX
Representative's Relationship/Reason Patient Unable to Sign

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:38 CST

Signature of Witness/Hospice Representative

November 25, 2025

Date

The Hospice furnished the addendum on:

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Jerilyn Henry.

I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

- I. I understand the Hospice's obligations are as follows:
 - A. The Hospice staff will provide instructions to me as needed.
 - B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.
 - C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.
- II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.
- III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:38 CST

November 25, 2025

Caregiver Signature

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:40 CST

November 25, 2025

Hospice Signature

Date

No Primary Caregiver Agreement

I, Jerilyn Henry, do agree and will comply with the following:

- Understand that I have an illness which has been diagnosed as appropriate for hospice care.
- Understand the goal of hospice care is to relieve, control or minimize symptoms to maintain optimal quality of life in patients with a life limiting illness. Treatment and diagnostic procedures which would cause increased discomfort without increased quality of life will be avoided.
- At the present time I live alone with no primary caregiver.
- When the Hospice Team determines that, due to disease progression or changes in my condition, it is unsafe for me to remain in my home alone, I agree to the following change of my environment. This may include contacting or hiring someone to provide 24-hour care to me.

1. _____
2. _____
3. _____
4. _____

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:57 CST

Patient Signature

November 25, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:41 CST

Witness

November 25, 2025

Date

Patient Choice Statement
(To be completed for all new patients/guardians)

I, Jerilyn Henry (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Homage Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:42 CST

November 25, 2025

Signature of Patient/Guardian

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:43 CST

November 25, 2025

Signature of Hospice Representative

Date

Hospice Wound Photography Consent

I, Jerilyn Henry _____ hereby consent and so authorize the taking
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 11-26-25

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:45 CST

November 25, 2025

Patient Signature (or other authorized signature)

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:46 CST

November 25, 2025

Witness Signature

Date

Daughter

Relationship, if other than patient

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:47 CST

Beneficiary/Patient/Authorized Representative Signature

November 25, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-11-25 15:54:48 CST

Hospice Agency Representative Signature

November 25, 2025

Date



HOMAGE
—HOSPICE—

Homage Hospice
8204 Elmbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name Jerilyn Henry Birth Date 01/18/1942
Address 720 Creek Valley Ct
City Allen State TX Zip 75002
Home Phone _____ Business Ph _____ Cell _____

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

****Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.****

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof, "Medical Records" includes the following:
Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Hospice Tools in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

E-SIGNED by Noel Starkweather
on 2025-11-25 17:07:51 CST

November 25, 2025

Patient/Authorized Signature

Date

Allergic to latex

**Mustang Creek Estates
Individualized Service Plan**

Resident Name: Serilynn Henry
Month of: November Page: 1 of 1 Date: 11/25/24

Basic Services

*Three meals per day, plus snacks as requested
*Daily housekeeping

*Weekly flat and bed linen laundry
*Social Activities and Exercise

Individual Needs:

Advanced Directives:

Medication Reminders

NO Ambulation aids If yes: NO Walker NO Cane NO Wheelchair
NO Dentures YES Eyeglasses NO Hearing Aides NO Prosthetic device
YES ADL assistance NO Toileting assistance NO Transfer assistance

Cognitive approach: Dementia, Alib Specifics of care: can get confused. Short term
Wandering: NONE
Mood/Behavior: Sad, Anxious, Loner
Social activities: Loner, Watches TV.
Communication: english clear.
Hearing: No hearing aides
Vision: Wears glasses
Skin: dry, fragile, bruises easily.
Ambulation: NO
Transfers: and
Sleep patterns: uses bathroom a lot ↑ 9:00a.m. ↓ 10:00p.m.
Continence: Continent
Eating habits: Not picky if upset doesn't want to eat. Coffee/cottage cheese
Oral health: Has her own teeth
Dressing: and remind her to change clothes.
Grooming: and
Showering: Refuses showers will Bird Bath.
Other:

By signing this document I certify the information I have provided in this assessment is factual and complete. I understand Mustang Creek Estates does not provide twenty-four hour one on one care or skilled medical care. I understand MCE does not employ nurses. I further agree not to hold Mustang Creek Estates liable for any information not disclosed during this assessment process.

Resident or Family Signature: X Ms. Serilynn Henry
Date: 11/25/24

Mustang Creek Estates Signature: Rebecca Covert

Date: 11/25/24



Resident Assessment

Resident Name: Jerilyn Henry Completed Date: 11/25/24
Completed By: Rebecca Covert Total Score: _____

1. **Date of Birth:**

1/18/21942

2. **Reason for Assessment:**

New Move In.

3. **Family Contact:**

David Henry

4. **Phone #:**

503-268-2460.

5. **Moving from (address):**

3628 Piedmont Dr, Plano TX 75075

6. **Diagnosis:**

Dementia. Alib.

7. **Any Known Drug Allergies:**

Latex.

8. **Hospitalizations within last 6 months:**

No.

9. **Cognitive: (circle all that apply)**

☒ A. Oriented to: Person
making: Independent
Moderately Impaired
I. Memory: Impaired Short Term

☒ B. Oriented to: Place

☒ E. Decision Making: Minimally Impaired

☐ G. Decision Making: Severely Impaired

J. Memory: Impaired Long Term

☒ C. Oriented to: Time

☐ D. Decision

☐ F. Decision Making:

☐ H. Memory Intact

10. **Comments:**

Short term



Resident Assessment

Resident Name: Jerilyn Henry Completed Date: 11/25/24
 Completed By: Rebecca Covert Total Score: _____

11. **Wandering/Alzheimers/Dementia History if any: (circle all that apply)**

☒ A. None ☐ B. Within the home ☐ C. Risk for elopement

12. **Comments:**

13. **Mood/Behavior/Psychosocial Issues: (circle all that apply)**

☒ A. Friendly & Cooperative ☐ B. Withdrawn ☒ C. Anxious ☒ D. Sad ☐ E. Aggressive
☐ F. Emotionally labile ☐ G. Depression History

14. **Comments:**

loner, watches T.V.

15. **Social: (circle all that apply)**

☒ A. Daily contact with relatives/friends ☒ B. Prefers to be alone

16. **Comments:**

Prefers to watch T.V.

17. **Communication: (circle all that apply)**

☒ A. Clear understand ☐ B. Able to be understood ☐ C. Able to call for assistance ☐ D. Difficult to understand
☐ E. Soft ☐ F. Slurred ☐ G. Aphasic ☒ H. Primary Language: English
☐ I. Primary Language: Not English

18. **Comments:**

19. **Hearing: (circle all that apply)**

☒ A. Intact ☒ B. Left Ear - Fair ☒ C. Right Ear - Fair ☐ D. Left Ear - Poor ☐ E. Right Ear - Poor
☐ F. Uses Hearing Aid - Left ☐ G. Uses Hearing Aid - Right ☐ H. With Hearing Aid: Good - Left
☐ I. With Hearing Aid: Good - Right ☐ J. With Hearing Aid: Poor - Left ☐ K. With Hearing Aid: Poor - Right
☐ L. Independent ☐ M. Reminders ☐ N. Assistance



Resident Assessment

Resident Name: Jerilyn Henry Completed Date: 11/25/24
 Completed By: Rebecca Curren Total Score: _____

20. **Comments:**

21. **Vision: (circle all that apply)**

A. Intact B. Fair - Left C. Fair - Right D. Poor - Left E. Poor - Right
 F. Legally Blind G. Glasses H. Good with Glasses I. Poor with Glasses

22. **Comments:**

Wears glasses.

23. **Skin: (circle all that apply)**

A. Good easily B. Dry C. Fragile D. Broken areas E. Rash F. Scars G. Bruises
 H. Edema I. Wounds J. Abrasions

24. **If yes to above, please describe**

Recurring cellulitis

25. **Apply Lotion (circle all that apply)**

A. Yes Assist B. No C. Independent D. Verbal Assist E. Standby Assist F. Physical

26. **Comments:**

Refuses

27. **Ambulation: (circle all that apply)**

A. Independent B. Cane C. Walker D. Wheelchair
 E. Other _____

28. **Comments:**



Resident Assessment

Resident Name: _____

Completed Date: _____

Completed By: _____

Total Score: _____

29. **Weakness:**

Weakness

30. **Sleep patterns: (circle all that apply)**

A. Sleeps through night

B. Up frequently

C. Naps during the day

31. **Preferred Rise Time:**

↑

9:00 A.M.

32. **Preferred Bed Time:**

↓

10:00 P.M.

33. **Comments:**34. **Continence: (circle all that apply)**

A. Continent of Bowel and Bladder

B. Incontinent: Needs reminders

C. Incontinent:

Needs assistance to toilet

D. Incontinent : Needs assistance with product

E. Able to

perform pericare:

F. Incontinent of Bladder

G. Incontinent of Bowel

H. Prone to

diarrhea I. Prone to constipation

35. **Comments:**

USES depends occasional accident.



Resident Assessment

Resident Name: Jerilyn Henry Completed Date: 11/25/24
 Completed By: Rebecca Court Total Score: _____

36. **Eating habits: (circle all that apply)**
 A. Good assist B. Fair C. Poor D. Eats independently E. Cut up food F. Physical
 G. Remind to meals

37. **Food Allergies: (circle one)**
 A. No B. Yes

38. **If yes, list:**
if upset does not want to eat.

Coffee
Cottage

39. **Special Diet: (circle all that apply)**
 A. No added salt B. No concentrated sweets C. Mechanical soft

40. **Comments:**
Not picky
Not a dessert eater.

41. **Oral Health: (circle all that apply)**
 A. Has own teeth B. Upper dentures C. Lower dentures D. Partial dentures
 E. Brushes teeth F. Requires assistance

42. **Comments:**

43. **Dressing: (circle one)**
 A. Independent B. Verbal Assistance C. Stand by Assistance D. Physical Assistance

44. **Comments:**
needs to remind her to
change her clothes.

45. **Grooming: (circle all that apply)**



Resident Assessment

Resident Name: Jerilyn Henry Completed Date: 11/25/24
 Completed By: Rebecca Covert Total Score: _____

☒ A. Independent

☐ B. Verbal assistance

☐ C. Standby assistance

☐ D. Physical assistance

46. **Comments:**

47. **Bathing/Showering (circle all that apply)**

☐ A. Independent

☐ B. Verbal assistance

☒ C. Standby assistance

☐ D. Physical assistance

☐ E. Prefers hydro jet tub

48. **Bathing/Showering Frequency**

refuses showers Bird Bath

49. **Comments:**

50. **Transferring: (circle all that apply)**

☒ A. Independent

☐ B. Stand by assistance

☐ C. two people

☐ D. Physical assist min

☐ E. Physical assist mod

☐ F. Physical assist max

51. **Transferring Equipment: (circle all that apply)**

☐ A. Sliding Board

☐ B. Hoyer Lift

☐ C. Other: _____

52. **Comments:**

53. **In the event of an emergency, can this person be transferred with the assistance of 1 or less? (circle one)**

☐ A. No

☒ B. Yes

54. **Medications: (circle all that apply)**

☐ A. Independent

☐ B. Self

☐ Administered



Resident Assessment

Resident Name: Jerilyn Henry Completed Date: 11/25/24
 Completed By: Rebecca Curren Total Score: _____

☒ C. Medication supervision/reminders drops ☐ G. Verbal assistance		☐ D. Family administers ☐ H. Stand by assistance		☐ E. Uses oxygen ☐ F. Eye	
55.	Medication Side Effects: (circle one) A. Yes B. No C. N/A <u>We give her med.</u>				
56.	Comments: <u>IBS — too much coffee is an issue</u>				
57.	Activity Preferences: (circle all that apply) A. Group ☒ B. One on one C. Alone				
58.	Describe types of leisure activities: <u>Watches T-V.</u>				
59.	Cardiac/Respiratory Status/Health Conditions/Preventative Health Needs (circle all that apply) A. Normal B. Chest Pain C. SOB at rest D. SOB on exertion E. Cough F. Pacemaker G. B/P Checks H. Smokes I. Independent J. Verbal Assistance K. Standby Assistance L. Physical Assistance				
60.	Comments: <u>has some stints Afib</u>				
61.	Additional Services Received/Special Treatments & Procedures (circle all that apply) ☒ A. None B. Home Health C. Rehabilitation D. Hospice E. Psychological F. Wound Vac G. Other: describe below				
62.	Other 				



Resident Assessment

Resident Name: Jerilyn Henry Completed Date: 11/25/24
Completed By: Rebecca Covert Total Score: _____

Individualized Service Plan:

1. **Review Date:**

2. **Resident Signature:**

3. **Date:**

4. **Family Member Signature:**

X M. Stammers

5. **Date:**

11/25/24

6. **Person Completing Assessment:**

Rebecca Covert

7. **Date:**

11/25/24

Patient	: Jerilyn Henry	Record Id	: PAT2852
Date of Birth	: Jan 18, 1942	Age / Gender	: 82 / F
Provider	: Dr. Sophia Rahman, MD	Date of Visit	: Jul 30, 2024
Visit Type	: New Patient Physical		

History of Present Illness

Ms Henry is a pleasant 82 year old Female presenting today for their annual exam.

Last Colonoscopy: Many years ago not doing anymore

Last Cologuard: No

Past Mammo and results: Decades ago declines

DEXA date and results: Declines

Immunization status (up to date, delayed, declines):

Last flu vaccine: Does not get flu shot

Last pneumonia vaccine: Declines

Shingles vaccine: unknown

MEDICAL HISTORY (List any current or past medical conditions) : Congestive heart failure

Cellulitis

Dementia

Skin cancer - on face

SURGICAL HISTORY(List any past surgeries / procedures) : Heart surgery 2012 - 2 stents

Neck/shoulder surgery - ??? No time or place remembered

Surgery on wound to lower left leg 5/2022 - sepsis leading to 3 months in the hospital

Allergies : Latex

SOCIAL HISTORY

Marital Status : Married

Occupation : Retired bookkeeper

How often do you wear a seatbelt while in the car? (always/sometimes/never) : Always

What kind of regular exercise do you do? : None

How often do you exercise? : None

Do you currently use drugs? : No

How much? (ex: daily, monthly, yearly) : --

Do you currently drink alcohol? : Yes

How much? (ex: daily, monthly, yearly) : 2-3 times per year - 1-2 glasses of wine

Do you currently use tobacco? (cigarettes, cigars, e-cigarettes, vapes, chewing tobacco, etc.) : No

How much? (ex: daily, monthly, yearly) :

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : Acute Visit - 15

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Nov 21, 2024

Chief Complaints

having dysuria

moving to Florida to a new care home and may need forms filled out about her medical history

History of Present Illness

Severity mild

Duration intermittent

Onset/Timing less than 1 week

Trend Worsening

Review of Systems

GENERAL: Denies fevers, weight gain or loss, night sweats or fatigue.

OPHTH.: Denies diplopia, blurred or double vision, Photophobia, discharge, or vision loss.

ENT: No abnormal nasal discharge or pain, no epistaxis, no mouth or throat discomfort.

CV: No reported chest pains, rapid or slow heartbeats, palpitations, or shortness of breath

PULM: No cough, shortness of breath, pleuritic chest pains, wheezing, or hemoptysis.

GI: Denies abdominal pain, nausea, vomiting, diarrhea, constipation, melena, or hematemesis.

GU: SEE HPI

HEME: No new bleeding or bruising tendency. No phlebitis or leg edema.

NEURO: No new weakness or paresthesias. No seizures, no alterations of consciousness, No headaches.

SKIN: No rashes or petechia, no discharge and no abnormal pruritis.

ALL: No abnormal congestion, no abnormal sneezing, no skin rashes, swelling, or pruritis.

PSYCH: No memory loss, depression, agitation, or suicidal thoughts.

ENDO: No excessive thirst or urination, no heat or cold intolerance

Health Vitals

Height: 5 ft 1 ins

Weight: 114 lbs 2 ozs

BMI: 21.56

BP: 132 / 74 mmHg

Pulse Rate: 76 bpm

Pulse Pattern: Regular

SPO2: 97 %

Physical Examination

GENERAL: The patient is alert Oriented x 3, and in no acute distress

HEENT: Normocephalic, atraumatic

CHEST: Chest structure normal with equal bilateral movements.

SKIN: Normal in color and in texture. No rashes or abnormal lesions noted.

PSYCH: Mood appears normal with no evidence of depression, agitation, or abnormal response.

In house UA

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : Acute Visit - 15

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Nov 21, 2024

A handwritten signature in black ink that reads "Sophia Rahman". The signature is written in a cursive style with a horizontal line underneath the name.

Dr. Sophia Rahman, MD

Electronically Signed By Dr. Sophia Rahman, MD on Nov 21, 2024 03:02 PM US/Central

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : New Patient Physical

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Jul 30, 2024

History of Present Illness

Ms Henry is a pleasant 82 year old Female presenting today for their annual exam.

Last Colonoscopy: Many years ago not doing anymore

Last Cologuard: No

Past Mammo and results: Decades ago declines

DEXA date and results: Declines

Immunization status (up to date, delayed, declines):

Last flu vaccine: Does not get flu shot

Last pneumonia vaccine: Declines

Shingles vaccine: unknown

MEDICAL HISTORY (List any current or past medical conditions) : Congestive heart failure

Cellulitis

Dementia

Skin cancer - on face

SURGICAL HISTORY(List any past surgeries / procedures) : Heart surgery 2012 - 2 stents

Neck/shoulder surgery - ??? No time or place remembered

Surgery on wound to lower left leg 5/2022 - sepsis leading to 3 months in the hospital

Allergies : Latex

SOCIAL HISTORY

Marital Status : Married

Occupation : Retired bookkeeper

How often do you wear a seatbelt while in the car? (always/sometimes/never) : Always

What kind of regular exercise do you do? : None

How often do you exercise? : None

Do you currently use drugs? : No

How much? (ex: daily, monthly, yearly) : --

Do you currently drink alcohol? : Yes

How much? (ex: daily, monthly, yearly) : 2-3 times per year - 1-2 glasses of wine

Do you currently use tobacco? (cigarettes,

cigars, e-cigarettes, vapes, chewing tobacco, etc.) : No

How much? (ex: daily, monthly, yearly) : --

Do you currently consume caffeine?

(tea, coffee, soda, etc.) : Yes

How much? (ex: daily, monthly, yearly) : Daily - 4-5 cups

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : New Patient Physical

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Jul 30, 2024

FAMILY HISTORY

FAMILY MEMBER: Mother

AGE (current/at death) : --
MEDICAL PROBLEMS : --

Father

AGE (current/at death) : --
MEDICAL PROBLEMS : --

Review of Systems

GENERAL: Denies fevers, weight gain or loss, night sweats or fatigue.
OPHTH.: Denies diplopia, blurred or double vision, Photophobia, discharge, or vision loss.
ENT: No abnormal nasal discharge or pain, no epistaxis, no mouth or throat discomfort.
CV: No reported chest pains, rapid or slow heartbeats, palpitations, or shortness of breath
PULM: No cough, shortness of breath, pleuritic chest pains, wheezing, or hemoptysis.
GI: Denies abdominal pain, nausea, vomiting, diarrhea, constipation, melena, or hematemesis.
HEME: No new bleeding or bruising tendency. No phlebitis or leg edema.
NEURO: No new weakness or paresthesias. No seizures, no alterations of consciousness, No headaches.
SKIN: No rashes or petechia, no discharge and no abnormal pruritis.
ALL: No abnormal congestion, no abnormal sneezing, no skin rashes, swelling, or pruritis.
PSYCH: No memory loss, depression, agitation, or suicidal thoughts.
ENDO: No excessive thirst or urination, no heat or cold intolerance

Health Vitals

Height: 5 ft 1 ins
Weight: 110 lbs 6 ozs
BMI: 20.85
Temperature: 97.6 F
BP: 148 / 86 mmHg
Pulse Rate: 68 bpm
Pulse Pattern: Regular
SPO2: 95 %

Physical Examination

GENERAL: The patient is alert Oriented x 3, and in no acute distress
HEENT: Normocephalic, atraumatic, PERRLA, EOMI. Ears clear with normal TMs. Naso & Oro-pharynx are clear.
NECK: Supple with no masses or adenopathy. Thyroid normal, without lesions or goiter. No carotid bruits.
PULMONARY: Clear to auscultation and percussion. No rales, rhonchi or wheezing. Breath sounds equal

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : New Patient Physical

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Jul 30, 2024

bilaterally.

CV: Regular rate and rhythm. S1, S2 heard, no S3. No rubs, murmurs, or gallops appreciated.

CHEST: Chest structure normal with equal bilateral movements. No deformities noted, no masses palpated.

GI: Soft, nontender and nondistended. Bowel sounds normal. No masses, no hepato or splenomegaly.

GU: No swelling rash or discharge noted. No CVA tenderness. No hernia.

EXT: FROM. No edema, cyanosis, or clubbing. Extremities symmetric w/no joint effusions. Pulses 2 + bilaterally.

NEURO: Alert and oriented X3. Motor strength equal. Sensory intact bilaterally. Gait is normal.

SKIN: Normal in color and in texture. No rashes or abnormal lesions noted.

PSYCH: Mood appears normal with no evidence of depression, agitation, or abnormal response.

In house UA

Leukocytes +15

Nitrite neg

Urobilinogen Normal

Protein Neg

PH 6

Blood Neg

Specific Gravity 1.020

Ketone neg

Bilirubin neg

Glucose neg

Assessment Notes

Reviewed medical, social and family history with the patient, as well as a current list of providers and prescriptions.

The patient was given personalized health advice including a list of risk factors and treatment options.

A Health Maintenance Sheet

with a checklist of recommended preventive care services was also given to the patient

Audit-C and PHQ 9 reviewed in detail

Pt encouraged to watch diet and exercise regularly to get this down to normal range.

-Your blood pressure is normal. Be sure to exercise regularly, try to manage stress and watch your sodium intake.

check labs today

f/u cardiology/neurology (Dr. Din soon)

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : New Patient Physical

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Jul 30, 2024

needs DERM EVAL ASAP

Diagnoses

Diagnosis	Code	Comments
Encounter for general adult medical examination without abnormal findings	Z00.00	
Encounter for screening for other disorder	Z13.89	
Body mass index [BMI] 20.0-20.9, adult	Z68.20	
Essential (primary) hypertension	I10	
Hyperlipidemia, unspecified	E78.5	
Unspecified atrial fibrillation	I48.91	--
Heart failure, unspecified	I50.9	
Atherosclerotic heart disease of native coronary artery without angina pectoris	I25.10	
Unspecified dementia without behavioral disturbance	F03.90	
Actinic keratosis	L57.0	

Treatments

Thank you for coming in for your Annual Wellness Exam today. Here is a summary of our goals for annual wellness and for screening.

1. I encourage a healthy diet , including low glycemic, low saturated fats, appropriate calories, nutrient dense, organic/pesticide free, plant based; I encourage healthy fat sources such as nuts, seeds, fish, fish oil supplements (I like Nordic Naturals), flax seed/oil, avocados, olives (if you don't have high blood pressure). Avoid saturated fats, especially from animal sources, as this has been correlated to higher rates of cardiac disease and colon cancer, especially red meats and meats preserved with nitrates.

2. Exercise: Current recommendations 30-60 min daily of cardio (sustained heart rate) and weights 2x/week

3. Recommended screenings per national guidelines are as follows:

A. Colonoscopy age 50 and then per findings. unless family history of colon cancer-then start earlier per provider--prior to the colonoscopy and in between colonoscopies, we advise a simple colon cancer screen called the FOBT (fecal occult blood test).

B. Mammo annually starting at age 40 unless strong fam hx of breast CA, then we advise starting at age 35 or sooner based on family history; high risk breast cancer patients may benefit for annual breast MRI screening for more accuracy/detail as well as referral to High Risk Breast Cancer Clinic where genetic counseling may be offered.

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : New Patient Physical

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Jul 30, 2024

If a mammogram was ordered for you today, your imaging order has been sent to CORA-Central Oregon Radiology. You may need to call them to schedule your appointment. Please allow 24 hours for them to process this order before scheduling. You may call them at (541) 382-9383. They should send me your results to review *generally* within 72 hours (business days), and I will share/review your results with you via the portal or at your follow up appointment.

*Most insurances cover the cost of a screening mammogram; however, we strongly advise you call your insurance company and CORA directly prior to scheduling your mammogram if cost is a concern or question. Further, if a lump is found on your routine exam, we cannot send an order for a routine preventative screening mammogram--we must send over an order for a diagnostic mammogram and ultrasound. This means that your insurance will not consider a diagnostic mammogram "routine" or "screening" and may apply the cost to your deductible. Again, the patient is strongly advised to be familiar with their insurance benefits and cost before scheduling their imaging.

C. PAP every 3 yrs if normal until age 65 for those with a cervix; no PAP needed after age 65 if normal; if PAP is abnormal, discuss with provider when your next one is due and what the follow up plan is. It typically takes about a week for us to get your PAP results back--we will notify you of results via the portal.

D. Self breast exam (SBE)----advise monthly checks and to set date on calendar for checking

E. Check your skin regularly for any irregular moles or skin changes and schedule exam with possible biopsy --annual dermatology appointment is advised if you have a family history of melanoma

F. Annual full eye exam with eye doctor is advised to screen for cancer, increased eye pressure (glaucoma), damage to small vessels or damage to the retina which can lead to early vision loss. Please get a full eye exam with eye doctor (not sure glasses check) annually.

G. Biannual dental is advised as dental health is related to heart health

H. Lung Cancer Screening: for anyone who has a smoking history, a chest CT to screen for lung cancer nodules is usually covered by insurance after age 50 and is advised

I. DEXA (bone density) screening: starting at age 65 unless there are other risk factors (such as taking chronic steroids or history of osteoporosis or history of frequent fractures)

4. Mental health: advised continued self-care, exercise, balanced diet. I like the phone app "CALM" for help with anxiety or insomnia or stress. We are also happy to refer you to a therapist if this is needed. You can find a list of therapists and other providers offering telehealth/zoom visits on our website.

Reviewed PHQ and GAD--depression and anxiety screening with counseling reviewed today if you completed your questionnaires. AUDIT/SBIRT drug/alcohol screening form likewise was reviewed today with counseling if completed.

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : New Patient Physical

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Jul 30, 2024

5. Sexual health: we encourage STI screening annually for sexually active adolescents/teens/young adults, and for all adults with any concerns or new/multiple partners. For concerns with painful sex or abnormal menses or postmenopausal sexual health, please schedule a follow up appointment.

6. Screening Labs--if ordered today, these will be shared to your portal upon provider review and we may ask you to schedule a follow up to review any abnormal results. Lab results can take anywhere from 2-7 business days to return, depending if labs had to be sent out of state to speciality labs.

We strongly advise you get labs drawn after talking to your insurance company to find out what they cover and when. We order the labs but have no control over the prices nor what percent of the cost your insurance will cover.

Your labs have been faxed to the lab. Please print/show the electronic requisition in your portal at the lab to ensure it matches the faxed order-- lab orders are missed as sometimes faxes are not transmitted in entirety. You don't need an appointment to get labs drawn, but we encourage you to be fasting (10-12 hours. You may have black coffee or water. The more hydrated you are, the more accurate. We will contact you via portal or phone with your lab results. Labs can take anywhere from 24-72 hours or longer to be resultated, depending if they were sent out or not. We will notify you of lab results via the portal or with a phone call. Please be aware that labs results may take 24-72 hours (sometimes longer) to arrive for review at our clinic.

**Annual screening labs generally include a complete blood cell count (CBC) to look for anemia, platelets, immune function; a comprehensive metabolic panel (to screen for kidney and liver health and prediabetes screening); thyroid hormone; vitamin D and iron levels; cholesterol panel, and sexual transmitted infection screening if at risk.

7. ASCVD score counseling: This score calculates your cardiovascular stroke risk score when lipids are received--please schedule a follow up appointment to review your score if getting lipids done today.

Your score and recommendations:

8. Immunization counseling:

We advise TDAP every 10 years, though some evidence shows that immunity can wane after 4-5 years and encouraged patients to get their booster shot more frequently. We also advise annual flu shot and your adult pneumonia shot if you are over 65 or younger than 65 with high risk criteria. We encourage COVID-19 booster when eligible. We advise Shingrix vaccine for anyone above the age of 50 (at your local pharmacy). We encourage HPV vaccine for young adults aged 18-26.

9. Sleep advise: sleep plays an integral role in metabolism/weight, headaches, heart health, mental health. We encourage sleep hygiene and 8 hours of sleep a night. If struggling with sleep, please set up follow up appointment to discuss further.

10. Skin care: We advise zinc or titanium only sunscreen to limit exposure to harmful, estrogenic chemicals

Patient : Jerilyn Henry
Date of Birth : Jan 18, 1942
Provider : Dr. Sophia Rahman, MD
Visit Type : New Patient Physical

Record Id : PAT2852
Age / Gender : 82 / F
Date of Visit : Jul 30, 2024

found in non-mineral sunscreens-reapply every 90 min, cover with long shirt and pants and hats; avoid prolonged sun exposure and tanning beds. We advise full derm check annually and we do a skin check during your annual wellness visit as well.

Follow up

Type	Notes	Date
Annual Visit	Annual visit with Dr. Sophia Rahman July 2025. Please call 972 782 2811 to schedule	in 12 months
Office Visit	Follow up visit with Dr. Sophia Rahman January 2025. Please call 972 782 2811 to schedule	in 6 months



Dr. Sophia Rahman, MD

Electronically Signed By Dr. Sophia Rahman, MD on Jul 30, 2024 10:35 AM US/Central

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/1/2025 10:00:00 PM
Date/time out: 12/1/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/1/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/2/2025 10:00:00 PM
Date/time out: 12/2/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/2/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/3/2025 10:00:00 PM
Date/time out: 12/3/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/3/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/4/2025 10:00:00 PM
Date/time out: 12/4/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/4/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/5/2025 10:00:00 PM
Date/time out: 12/5/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/5/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/8/2025 10:00:00 PM
Date/time out: 12/8/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/8/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/9/2025 10:00:00 PM
Date/time out: 12/9/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/9/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/10/2025 10:00:00 PM
Date/time out: 12/10/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Assist

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/10/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER VISIT NOTE

HOMEMAKER VISIT NOTE

Date/time In: 12/11/2025 10:00:00 PM
Date/time out: 12/11/2025 10:10:00 PM

Hair Care

Comb/brush Assist

Nutrition

Prepared/Served Breakfast: Total
Prepared/Served Lunch: Total
Prepared/Served Dinner: Total
Offered food, fluids: Assist

Special Needs/Requests

Provided social/emotional support: Total

Home Management

Changed bed linens: Total
Washed patient's laundry: Total
Cleaned patient's room/area: Total
Washed patient's dishes: Total
Grocery shopped for patient: Total
Performed errands for patient: Total
Cleaned patients equipment: Total
Cleaned filter on oxygen concentrator: Total

Miscellaneous

Reminded patient to take meds: Total

Safety Precautions (see assignment sheet for specific precautions)

- ☒ Aspiration precautions
- ☒ Behavioral precautions
- ☒ Bleeding precautions
- ☒ Diabetic precautions
- ☒ Fall precautions
- ☒ Fragile bone precautions
- ☒ Fragile skin precautions
- ☒ Opioid precautions
- ☒ Oxygen precautions
- ☒ Seizure precautions
- ☒ Wandering precautions

Date: 12/11/2025

Electronically signed by: MORGAN STOVALL

Patient: JERILYN HENRY
Medical Record: #97137

HOMEMAKER ASSIGNMENT AND PLAN OF CARE

Goals

- ☒ Safe and effective personal care and hygiene
- ☒ Patient clean and comfortable
- ☒ Respite for family
- ☒ Socialization/support
- ☒ Environment safe and clean

Patient Information

- *General Condition*
 - ☒ Symptoms managed well
- *Mental/Behavioral Status*
 - ☒ Alert
 - ☒ Withdrawn
 - ☒ Confused
 - ☒ Forgetful

Nutrition

- ☒ Feeds self
- Other: as tolerated

Functional Limitations

- ☒ OOB with assist

Assistive Devices/Equipment Used During Patient Care

- ☒ Walker
- ☒ Glasses

Elimination

- ☒ Toilet
- ☒ Incontinent
 - ☒ Bladder
 - ☒ Bowel

Senses

- ☒ Hard of hearing
- ☒ Poor vision

Living Arrangements

☒ Lives in residential facility

Describe: Assisted Living

Hygiene and grooming

• Hair Care

☒ Comb/brush

Total/Assist: Assist

Frequency: 5

• Special Needs/Requests

☒ Provide social & emotional support and companionship

Total/Assist: Assist

Frequency: 5

Electronically signed by: TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE CARE PLAN

Goals:

- ☒ Safe and effective personal care and hygiene
- ☒ Patient clean and comfortable
- ☒ Respite for family
- ☒ Socialization/support
- ☒ Environment safe and clean

Update #1

Date: 11/28/2025 5:00 AM

Describe patient condition requiring HA interventions: Pt a/o x 2 with confusion ambulates with FWW Needs Asst for 4/6 ADLs

Patient Info

General Condition

- ☒ Uncontrolled symptoms
Describe: cognitive decline needs assistance

Mental/Behavioral Status

- ☒ Alert
- ☒ Withdrawn
- ☒ Confused
- ☒ Forgetful

Nutrition

- ☒ Diet
Describe: as tolerated
- ☒ Feeds self

Functional Limitations

- ☒ OOB with assist

Assistive Devices/Equipment Used During Patient Care

- ☒ Walker
- ☒ Glasses

Elimination

- ☒ Toilet
- ☒ Incontinent

- ☒ Bladder
- ☒ Bowel

Senses

- ☒ Hard of hearing
- ☒ Poor vision

Living Arrangements

- ☒ Lives in residential facility
- Describe: asst living facility

Interventions

Pain Management

- ☒ Notify nurse if patient exhibits any verbal or non-verbal signs of pain or symptoms

Bathing

- ☒ Assist to dress
- Frequency: 5xw
- ☒ Shower
- Frequency: 3xw
- Instructions: Assist with shower

Hair Care

- ☒ Comb/brush
- Frequency: 5xw
- ☒ Shampoo/style
- Frequency: 2xw

Skin Care

- ☒ Inspect skin; check pressure areas
- Frequency: 5xw
- ☒ Call nurse if new skin problems are noted
- Frequency: 5xw
- ☒ Shave
- Frequency: 1xw whiskers
- Instructions: as needed
- ☒ Trim nails
 - ☒ Fingers
 - ☒ Toes

Frequency: 1xw

- ☒ Clean nails
- ☒ Fingers
- ☒ Toes

Frequency: 3xw

- ☒ Perineal care
- Frequency: 5xw

Foot Care

- ☒ Bathe/inspect
- Frequency: 3xw

Nutrition

- ☒ Offer food, fluids
- Frequency: 5xw

Special Needs/Requests

- ☒ Provide social & emotional support and companionship
- Frequency: 5xw

Elimination

- ☒ Incontinent care/diapers/briefs
- Frequency: 5xw
- Instructions: pullups
- ☒ Record most recent BMs reported
- Frequency: 5xw

Home Management

- ☒ Bed linen change
- Frequency: 1xw
- Instructions: and when soiled

Safety Precautions

- ☒ Safety precautions assigned

Date: 11/28/2025 5:00 AM
Electronic Signature: JOANNIE ESTES RN

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/1/2025 1:00:00 PM
Date/time out: 12/1/2025 1:41:00 PM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/1/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/1/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/2/2025 1:00:00 PM
Date/time out: 12/2/2025 1:50:00 PM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/2/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/2/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/3/2025 10:24:00 AM
Date/time out: 12/3/2025 11:14:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/3/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/3/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/4/2025 10:02:00 AM
Date/time out: 12/4/2025 10:47:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/4/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/4/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/5/2025 10:21:00 AM
Date/time out: 12/5/2025 11:10:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/5/2025 12:00:00 AM

Home Management

☒ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/5/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/8/2025 3:29:00 PM
Date/time out: 12/8/2025 4:17:00 PM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/8/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/8/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/9/2025 9:35:00 AM
Date/time out: 12/9/2025 10:17:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/9/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/9/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/10/2025 9:04:00 AM
Date/time out: 12/10/2025 9:36:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/10/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): She refused shower

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A
Date: 12/10/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/11/2025 9:35:00 AM
Date/time out: 12/11/2025 10:17:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/11/2025 12:00:00 AM

Home Management

☒ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/11/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/12/2025 10:41:00 AM
Date/time out: 12/12/2025 11:32:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/12/2025 12:00:00 AM

Home Management

☒ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/12/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/13/2025 4:03:00 PM
Date/time out: 12/13/2025 4:20:00 PM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/13/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/13/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/14/2025 2:20:00 PM
Date/time out: 12/14/2025 2:35:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☐ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Shaved
- ☐ Trimmed fingernails
- ☐ Trimmed toenails
- ☐ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Nutrition

- ☐ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☐ Provided incontinent care/changed diaper/briefs

☐ Recorded most recent BM (during visit)

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☐ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/14/2025

Electronically signed by: AMARACHI GRACE OKANUME CNA RN

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/15/2025 9:11:00 AM
Date/time out: 12/15/2025 9:55:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/15/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/15/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/16/2025 9:26:00 AM
Date/time out: 12/16/2025 10:12:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/16/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/16/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/17/2025 10:22:00 AM
Date/time out: 12/17/2025 11:10:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/17/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/17/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/18/2025 9:05:00 AM
Date/time out: 12/18/2025 9:50:00 AM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/18/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/18/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/19/2025 2:30:00 PM
Date/time out: 12/19/2025 3:24:00 PM

Pain Management

- ☒ No evidence of pain
- ☒ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☒ Shower

Hair Care

- ☒ Comb/brush
- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Shaved
- ☒ Trimmed fingernails
- ☒ Trimmed toenails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/19/2025 12:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☒ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/A

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/A

Date: 12/19/2025

Electronically signed by: VIRGINIA RAMIREZ CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/20/2025 5:20:00 PM
Date/time out: 12/20/2025 5:50:00 PM

Pain Management

- ☐ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☐ Shaved
- ☐ Trimmed fingernails
- ☐ Trimmed toenails
- ☐ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/20/2025 8:00:00 AM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☐ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): NA

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: NA

Date: 12/20/2025

Electronically signed by: TRACI MACHELLE MONTGOMERY CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/21/2025 5:20:00 PM
Date/time out: 12/21/2025 5:50:00 PM

Pain Management

- ☐ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Assisted patient to dress
- ☐ Shower

Hair Care

- ☒ Comb/brush
- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☐ Shaved
- ☐ Trimmed fingernails
- ☐ Trimmed toenails
- ☐ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Nutrition

- ☒ Offered food, fluids

Special Needs/Requests

- ☒ Provided social/emotional support

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

☒ Recorded most recent BM (during visit)

Date/time: 12/21/2025 5:00:00 PM

Home Management

☐ Changed bed linens

Safety Precautions (see care plan for specific precautions)

☐ Safety precautions followed

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): NA

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: NA

Date: 12/21/2025

Electronically signed by: TRACI MACHELLE MONTGOMERY CNA

Patient: JERILYN HENRY
Medical Record: #97137

HOSPICE ITEM SET - ADMISSION

Section A: Administrative Information

A0050. Type of record

1. Add new record

Patient / Admission Information

A0205. Site of service at admission

01. Hospice in patient's home/residence

A0220. Admission date

11/26/2025

A0245. Date initial nursing assessment initiated

11/26/2025

A0500A. Patient first name

JERILYN

A0500C. Patient last name

HENRY

A0550. Patient zip code

75002-5294

A0600A. Social Security Number

321-34-4786

A0600B. Patient Medicare/railroad insurance number

4E46RK5CU88

☒ N. Patient is not a Medicaid recipient

A0800. Gender

2. Female

A0900. Birthdate

1/18/1942

Race/Ethnicity

☒ A1000F. White

Payor Information

☒ A1400A. Medicare (FFS)

Admitted From

A1802. Admitted from

01. Community residential setting (e.g., private home/apt., board/care, assisted living, group home,

Section F: Preferences

F2000A. Was patient asked about CPR

1. Yes, and discussion occurred

F2000B. Date asked about CPR

11/26/2025

F2100A. Was patient asked about treatments other than CPR

1. Yes, and discussion occurred

F2100B. Date asked about treatment other than CPR

11/26/2025

F2200A. Was patient asked about hospitalization

1. Yes, and discussion occurred

F2200B. Date asked about hospitalization

11/26/2025

F3000A. Was patient asked spiritual/existential concerns

1. Yes, and discussion occurred

F3000B. Date asked about spiritual/existential concerns

11/26/2025

Section I: Active Diagnoses

I0010. Principal diagnosis

99. None of the above

Section J: Health Conditions

Pain

J0900A. Was patient screened for pain

1. Yes

J0900B. Date of first screening for pain

11/26/2025

J0900C. Patient's pain severity was

1. Mild

J0900D. Type of standardized pain tool used

4. Staff observation

J0905. Is pain an active problem for the patient?

0. No

Respiratory Status

J2030A. Was patient screened for shortness of breath 1. Yes
J2030B. Date of first screening for shortness of breath 11/26/2025
J2030C. Did screening indicate the patient had shortness of breath 1. Yes
J2040A. Was treatment for shortness of breath initiated 1. No, patient declined treatment

Section N: Medications

N0500A. Was scheduled opioid initiated or continued 0. No
N0510A. Was PRN opioid initiated or continued 0. No
N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes") 2. Yes
N0520B. Date bowel regimen initiated or continued 11/26/2025

Section Z: Record Administration

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.
Z0500B. Date of signature verifying record completion 11/26/2025

Electronically signed by: JOANNIE ESTES RN

Patient: JERILYN HENRY
Medical Record: #97137

COMFORT KIT ORDERS

All of the patient’s medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring

- ☒ Morphine Sulfate, 100mg/5ml = 20 mg/ml, 0.25-1ml, Sublingual, Q1-2 hours prn, Pain/SOB - #120mL, 00054040441
- ☒ Lorazepam Liquid, 1.75mg/ml, 0.5-1 mg, PO/SL, q4 hrs prn, Anxiety/SOB - #30mL / RF: 5, 00054353244
- ☒ Atropine sulfate oral Soluntion, 0.75%, 1-2 drops, Take Sublingually, every 2 hours as needed, for excess secretions - #15mL / RF: 11
- ☒ APAP 650MG SUPPS, 650 mg, 1 suppository, unwrap and insert Rectally, Q 4 HRS PRN, Pain/Fever - #4 / RF:11 , 00927067812
- ☒ Bisacodyl suppository, 10 mg, 1 suppository, unwrap and insert Rectally, Q DAY PRN, Constipation - #3 / RF:11 , 00363010908
- ☒ Zofran ODT, 1 tablet, 4mg, dissolve PO/SL, Q 8 HRS PRN, nausea/vomiting - #6 / RF:11
- ☒ Senna, 8.6mg, 1 Tab, PO, Daily, Constipation
- ☒ Oxygen, na, 2-4 liters, intranasally, PRN, Shortness of breath

Ordering physician: DIEGO F RESTREPO MD
Date: 11/26/2025

Electronically signed by: TRACY KING RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: JERILYN HENRY
Medical Record: #97137

MULTIPLE MEDICATIONS FORM

Multiple Medications Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Medications:

Start Date	Discontinue Date	Ordering Physician	Reviewing Physician	Medication	Unit strength	Dose	Form	NDC	Route	Frequency	Indication	Associated diagnosis	Days Supply	Is medication covered by hospice?	If not covered, reason medication is not covered
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	aspirin 81 MG Chewable Tablet	81 mg	1 tab	Chewable Tablet	00113025968	po	1 tab Daily	analgesics	I50.22 - Chronic systolic (congestive) heart failure (Primary)	15	Yes	
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	ubidecarenone 100 MG Oral Capsule [Q-Sorb Co Q-10]	100 mg	1 tab	Oral Capsule	74312004886	Oral Pill	daily AM	HTN	I50.22 - Chronic systolic (congestive) heart failure (Primary)	15	No	supplemental
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	apixaban 2.5 MG Oral Tablet [Eliquis]	2.5 mg	1 tab	Oral Tablet	00003089321	Oral Pill	bid	blood thinner	I50.22 - Chronic systolic (congestive) heart failure (Primary)	15	No	Not covered by hospice
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	lisinopril 10 MG Oral Tablet	10 mg	1 tab	Oral Tablet	00093111310	Oral Pill	daily	HTN	I50.22 - Chronic systolic (congestive) heart failure (Primary)	15	Yes	
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	24 HR metoprolol succinate 100 MG Extended Release Oral Capsule	100 mg	1 tab	Extended Release Oral Capsule	63304001030	Oral Pill	am	HTN	I50.22 - Chronic systolic (congestive) heart failure (Primary)	15	Yes	
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	quetiapine 25 MG Oral Tablet [Seroquel]	25 mg	1 tab	Oral Tablet	00310027510	po	BID	antipsychotic	G31.1 - Senile degeneration of brain, not elsewhere classified (Related)	15	Yes	
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	sertraline 100 MG Oral Tablet	100 mg	1 tab	Oral Tablet	00143965430	Oral Pill	am	Depression	G31.1 - Senile degeneration of brain, not elsewhere classified (Related)	15	Yes	
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	donepezil hydrochloride 5 MG Oral Tablet [Aricept]	5 mg	1 tab	Oral Tablet	49999075330	Oral Pill	HS	Behaviors	G31.1 - Senile degeneration of brain, not elsewhere classified (Related)	15	Yes	
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	lovastatin 10 MG Oral Tablet	10 mg	1 tab	Oral Tablet	00093092606	Oral Pill	HS	HLD		15	No	not covered by hospice
11/26/2025		Diego Restrepo	DIEGO F RESTREPO MD	acetaminophen 500 MG Oral Tablet [Tylenol]	500 mg	1 tab	Oral Tablet	00045044905	po	QID PRN	mild pain	I50.22 - Chronic systolic (congestive) heart failure (Primary)	15	Yes	

Date: 11/26/2025

Electronically signed by: JOANNIE ESTES RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: JERILYN HENRY
Medical Record: #97137

DISCONTINUE MEDICATION FORM

Discontinue Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start Date: 11/26/2025
Medication: quetiapine 25 MG Oral Tablet [Seroquel]
Unit Strength: 25 mg
Dose: 1 tab
Route: po
Frequency: BID
Indication: antipsychotic
Discontinue Date: 12/4/2025
Ordering Physician: Dr. Diego Restrepo
Reviewing Physician: DIEGO F RESTREPO MD

Electronically signed by: JOANNIE ESTES RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: JERILYN HENRY
Medical Record: #97137

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 12/4/2025
Ordering Physician: Dr.. Diego Restrepo
Reviewing Physician: DIEGO F RESTREPO MD
Medication: divalproex sodium 125 MG Delayed Release Oral Tablet [Depakote]
Unit strength: 125 mg
Dose: 1 cap
Form: Delayed Release Oral Tablet
NDC: 00074732513
Route: Oral Pill
Frequency: BID
Indication: behaviors
Associated diagnosis: G31.1 - Senile degeneration of brain, not elsewhere classified (Related)
Days Supply: 15
Is medication covered? Yes

Electronically signed by: JOANNIE ESTES RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: JERILYN HENRY
Medical Record: #97137

COVID-19 ASSESSMENT

COVID-19 ASSESSMENT

This assessment tool supports you in identifying patients at risk for COVID-19 and helps guide clinicians in clinical decision making.

Symptoms

Note: Symptoms may appear 2-14 days after exposure.

☒ None

Travel History

***Updated information on affected countries can be found at: <https://www.cdc.gov/coronavirus/2019-ncov/travelers/index.html>*

Action Taken

☒ None indicated

***Please refer to the CDC COVID-19 website for ongoing updates and guidance: <https://www.cdc.gov/coronavirus/2019-ncov>*

COVID-19 TESTING

Patient testing completed? No

Date: 11/26/2025

Electronically signed by: TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

LCD - CARDIAC

LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

Worsening of Clinical Status

- ☒ Recurrent or intractable infection; pneumonia, sepsis, UTI
- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss
- ☒ Dysphagia leading to recurrent aspiration and/or decreased oral intake documented by decreasing food portion consumption

Worsening of Symptoms

- ☒ Dyspnea with increasing respiratory rate
- ☒ Nausea, vomiting poorly responsive to treatment
- ☒ Intractable diarrhea

Worsening of Signs

- ☒ Edema
- ☒ Weakness
- ☒ Change in level of consciousness

- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs
- ☒ Increased ER visits, hospitalizations, or physician visits related to the hospice primary diagnosis

Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ Ischemic heart disease
- ☒ Dementia

Cardiac

NEW YORK HEART ASSOCIATION CLASS (Heart disease)

- ☒ Class III (Moderate) Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes fatigue, palpitation, or dyspnea.

HEART DISEASE

Patients should meet criteria in numbers 1 and 2. Supporting documentation is found in no. 3.

- ☒ 1: Patient has already been optimally treated with vasodilators unless contraindicated AND is not a candidate for surgery or has refused surgery

3: Any of the following will help support the diagnosis but is not required:

- ☒ **Treatment resistant symptomatic supraventricular or ventricular arrhythmias.**
- ☒ **History of unexplained syncope**

Date: 11/26/2025 1:31:00 PM

Electronically signed by: TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

LCD - GENERAL DECLINE IN CLINICAL STATUS

LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

Worsening of Clinical Status

- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss

Worsening of Signs

- ☒ Edema
- ☒ Weakness

- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs

Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ CHF
- ☒ Diabetes Mellitus
- ☒ Dementia

Date: 11/26/2025 1:32:00 PM

Electronically signed by: TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

NURSING VISIT NOTE (Abridged)

NURSING VISIT NOTE (Abridged)

Date/Time In: 11/28/2025 8:55:00 AM
Date/Time Out: 11/28/2025 9:25:00 AM
Type of visit: Scheduled
COVID-19 screening performed? Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Narrative: Hospice Tuck-In Visit Narrative

Patient is an 83-year-old Caucasian female, newly admitted to routine home hospice care. She is a DNR and is on service for chronic congestive heart failure. Additional diagnoses include senile degeneration of the brain, cellulitis, mixed incontinence, dementia, weakness, and history of falls.

Upon arrival, the patient was finishing breakfast. She typically eats 75–100% of her meals three times daily. She was assisted with ambulation using a four-wheel walker back to her room. Patient is known to be prone to wounds on her bilateral ankles. During assessment, she was noted to have an area on the left ankle consistent with bumping into an object. Caregiver reported a possible fall; the patient denies any fall. She is exhibiting increased confusion and disorientation, which the daughter reports has been progressively worsening.

Patient sat on the edge of the bed without difficulty. Assessment completed:

Lungs: Clear but diminished throughout; occasional irregularity noted, consistent with known atrial fibrillation per daughter.

Heart: Irregular at times.

Abdomen: Soft, non-tender; bowel sounds x4.

Last bowel movement: Yesterday.

Infection status: Recently started on Ciprofloxacin, day 2, for a suspected UTI—ordered prior to hospice admission.

Pain: Denies pain.

Sleep: Sleeps 8–10 hours nightly.

Mobility: Ambulates with four-wheel walker; high fall risk due to confusion and weakness.

Skin: Fragile skin; slow to heal. History of cellulitis in 2022, which progressed to sepsis. Ankles remain high-risk for recurring wounds.

Patient wears glasses. Education provided to caregiver and family on hospice philosophy, disease progression, monitoring for increasing confusion, fall prevention, skin integrity, and CHF symptom management. Reinforced the importance of observing changes in mental status and wound development.

POA is the husband, though he is currently in rehab recovering from hip replacement. Daughter present and assisting with care.

Plan to continue monitoring patient’s decline, skin integrity, wound onset, infection status, and cognitive changes. Will follow up closely.

Last bowel movement: 11/27/2025

Pain: 0

PRN medication usage last 24 hours: 0

Need medication refills: No

Discussion with caregiver/POA/facility educated on pt decline more confusion redirect when needed, monitor decline call hospice

staff:

Date: 11/28/2025

Electronically signed by: JOANNIE ESTES RN

Patient: JERILYN HENRY
Medical Record: #97137

NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

Date/Time in:12/1/2025 10:27:00 AM

Date/Time out:12/1/2025 10:28:00 AM

Purpose of progress note:IDG Narrative

IDG Patient Name: Jerilyn Henry
Narrative Age/Sex/Race: 83-year-old Caucasian female
Date of Admission: 11/26/2025
Level of Care: Routine Home Care
Primary Diagnosis: I50.22 - Chronic systolic (congestive) heart failure
Related Comorbidities: G31.1 - Senile degeneration of brain, not elsewhere classified, I48.91 - Unspecified atrial fibrillation, L03.90 - Cellulitis, unspecified, N39.46 - Mixed incontinence, F03.90 - Unsp dementia, unsp severity, without beh/psych/mood/anx, R53.1 - Weakness, Z91.81 - History of falling, R60.9 - Edema, unspecified, R19.7 - Diarrhea, unspecified, R06.03 - Acute respiratory distressUnrelated Conditions (if applicable): Z85.831 - Personal history of malignant neoplasm of soft tissueCertifying Physician: Dr. Diego Restrepo
POA/Decision Maker: Step Daughter – Noelle Henry/ David Henry Spouse
Advance Directives: DNR;
Allergies: NKDA, Latex Natural RubberCurrent Scales:PPS: 50%KPS: 50%Mid-Arm Circumference (LMAC): 22
FAST (if dementia): 6E
NYHA Classification: Class IV (SOB at rest)SubjectivePast Medical History / DeclineThe patient has a significant cardiac history, including heart surgery with two stents in 2012 and a history of atrial fibrillation, although she was in a regular rhythm at today's visit. She carries a diagnosis of dementia, which has notably worsened over the last two weeks, with increased confusion, withdrawal, reduced ambulation, and poor safety awareness. She is a high fall risk and requires redirection and reminders for most tasks.Patient has a prior history of cellulitis to the leg requiring hospitalization and surgery in 2022 following a fall. She has a history of facial skin cancer and has required treatment in the past. She reports dysuria, periodic diarrhea, and decreased appetite with fatigue and generalized weakness. She reports drinking wine 2–3 times a year.Patients and family verbalize understanding of hospice philosophy and desire comfort-focused care.Objective
General: Cachectic, Pt SOB at rest SAT 89%on RA she refuses Oxygen. Lungs are clear, diminished throughout.
Neuro: A&O x2 but fatigued and slow to respond. Has a blank stare hoh.
Respiratory: Respirations 17 /min at rest; diminished breath sounds mid to lower lobes throughout; O₂ saturation 88-89% on RA, She refuses Oxygen at this time.
Cardiac: HR 71, regular; BP 152/87 Pt with 2 stents placed in 2012, she has Afib
GI: Soft, non-tender; bowel sounds present; reports decreased appetite (50-75% of 2 meals). She may skin breakfast drinking coffee only
GU: Incontinent of urine just started on pull ups, toileting assistance is needed.
Skin: Dry, fragile, no open areas. Scab to left ankle noted d/t previous fall per cg 10 days ago
Extremities: Weak grip, generalized muscle wasting; NO lower-extremity edema.
Mobility: Pt ambulates with FWW high fall risk, weak needs assistance for safety.
Weight Change: unknown last weight per documentation was noted to be 114 lbs a year ago with BMI of 21.56.
Functional Status
Current PPS and KPS are both 50%. She ambulates short distances with a four-wheel walker, but mobility has declined, and she spends increasing time in her room alone. She becomes easily confused, responds “huh?” frequently, and demonstrates poor short-term memory recall. She requires reminders to change clothes and may wear the same clothing for days. She resists showers and prefers “bird baths”; however, she is scheduled for three showers weekly with aide assistance.She is increasingly lethargic and fatigued, stating she “doesn’t sleep,” although observation indicates she naps frequently, approximately 10 hours cumulatively per day.Nutrition / IntakeShe eats 1–2 meals per day, consuming 50–75% of offered meals. She eats smaller portions at breakfast. She drinks a significant amount of coffee and reports occasional diarrhea. No edema noted.
Respiratory / CardiovascularDuring this assessment, oxygen saturation was 88–89% on room air. She is refusing supplemental oxygen at this time. Lung sounds are shallow and clear/diminished throughout. Heart rhythm regular on assessment.
GI / GUAbdomen is soft and non-tender. She is unsure of her last bowel movement but estimates “two days ago.”
Dysuria reported. No bladder distention noted.
Psychosocial / BehavioralPatient is a loner, withdrawn, and prefers to stay to herself. She demonstrates increased anxiety and cognitive decline. She is forgetful and often responds to questions with “huh?” before attempting to answer.She takes quetiapine twice daily for behaviors and mood stabilization.Her husband is currently hospitalized for a hip fracture, which may be contributing to emotional withdrawal and decline.
SpiritualShe identifies as Baptist and has Bibles placed throughout her room. She finds comfort in her faith.
SafetyHigh fall risk
Poor safety awareness Poor short-term memoryIncreased confusion Declining mobility Requires repeated cues and reminders
Assessment SummaryThe patient appears to be in overall decline, with worsening dementia, increasing weakness, reduced oral intake, increased fatigue, poor hygiene, and decreased ability to perform ADLs independently. Her oxygen

saturation remains low on room air, and she refuses supplemental oxygen. Cognitive decline is progressing, and functional status has decreased to PPS/KPS 50%. Family is aware of her decline, agrees with the hospice plan of care, and is supportive at this time. Ongoing education has been provided regarding disease progression, fall risk, and safety needs. Plan Continue routine home care hospice services Monitor cognitive decline, nutrition, hydration, bowel habits Reinforce fall precautions and safety Encourage oxygen use as tolerated Provide aide support for hygiene and showers Provide emotional and spiritual support Continue monitoring for changes in condition, symptoms, or needs Clinical

Summary Statement (CMS Required) Based on clinical findings, performance scale decline, respiratory compromise, progressive weight loss, increased dependence with ADLs, and overall functional decline, the patient demonstrates a terminal trajectory consistent with end-stage COPD. It is the nurse's clinical judgment that the patient has a life expectancy of six months or less should the disease follow its normal course. Hospice care is appropriate and medically necessary.

Date: 12/1/2025

Electronically signed by: JOANNIE ESTES RN

Patient: JERILYN HENRY
Medical Record: #97137

LVN NURSE VISIT

LVN NURSE COMPREHENSIVE VISIT

Date/Time in: 12/3/2025 2:31:00 PM
Date/Time out: 12/3/2025 3:01:00 PM
COVID-19 screening performed? Yes
Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

- *Temperature*
Type of temperature taken: Temporal:
Temporal temperature: 98.2
- *Respiratory Rate*
Respiratory rate (respirations per minute): 18
- *Heart Rate*
Radial pulse: 76
- *Blood Pressure*
☒ Sitting
Systolic blood pressure: 126
Diastolic blood pressure: 62
- *Pedal Pulses*
☒ Present
☒ Right
☒ Left

Current Mental Status

- ☒ Alert
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 0

GI Elimination

Date of last bowel movement: 12/3/2025
Bowel Pattern regular
Bowel sounds, # of quads: 4

Respiratory

☒ **Clear**
Describe location, inspiratory/expiratory: all lobes clear
☒ **Dyspnea with exertion**

Mobility

☒ **Ambulatory**
Steady/unsteady gait: Unsteady gait
☒ **Poor endurance**
☒ **Weakness/fatigue**

Skin

Skin temperature: warm
Skin turgor: poor

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

- *Instructed on policies and procedures related of use and disposal of controlled drugs in the home*
 - ☒ Patient
 - ☒ Caregiver
- *Teaching provided during visit*
 - *Method(s)*
 - ☒ Verbal instruction
 - *Outcome(s)*
 - ☒ Verbalized understanding
- *Instructed on infection control procedures*
 - ☒ Patient
 - ☒ Caregiver
- ☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Additional Information/Comments: Patient assessed, alert and without signs of acute distress. Respirations even, pain reported as baseline, and skin warm and dry. Medications reviewed and taken as ordered caregiver said patient doesn't need any refills. Will continue routine and supportive care.

Date: 12/4/2025

Electronically signed by: TANEKA DAVIS LVN

Patient: JERILYN HENRY
Medical Record: #97137

NURSING VISIT NOTE (Abridged)

NURSING VISIT NOTE (Abridged)

Date/Time In: 12/4/2025 10:00:00 AM
Date/Time Out: 12/4/2025 10:30:00 AM
Type of visit: PRN
COVID-19 screening performed? Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Narrative: PRN Visit Narrative

PRN visit completed for an 83-year-old Caucasian female currently on hospice service for chronic systolic congestive heart failure and senile degeneration of the brain. Family requested a PRN visit due to increased agitation, behavioral changes, and the patient “not acting like herself.”

The patient recently completed a course of Ciprofloxacin for a UTI prior to hospice admission. Upon assessment, the patient presented with irregular heart rate, hyperactive bowel sounds, and significant short-term memory impairment. She repeatedly forgot that she had already eaten breakfast and was observed ambulating with her walker into the kitchen searching for food.

Patient was highly agitated, accusatory, and difficult to redirect. She verbalized that people were “pouncing on her” and needed to “back off,” repeating this several times. She required frequent redirection with minimal effect. Despite this, the patient reported that she “slept well last night.”

Physical assessment revealed cold hands, clear but diminished lung sounds, and no acute respiratory distress. Daughter noted a possible developing runny nose. Per aides, the patient did eat breakfast this morning.

Family reports she is spending more time in remote memory, referencing past husbands; the patient has been married five times and is now discussing previous spouses. She is also expressing delusional jealousy, accusing her current husband (married 36 years), who recently had hip surgery and is recovering away from the home, of having a girlfriend.

Family states patient becomes more agitated when taking quetiapine. Review of medication history shows she began quetiapine 25 mg BID in June. After consultation with hospice medical director, quetiapine has been discontinued.

New order:

Depakote 125 mg PO BID for agitation and behavioral disturbances.

Education provided to family regarding medication changes, expected onset of effect, safety, and continued monitoring of behaviors. Encouraged family to call hospice with any further concerns or changes in condition.

Patient remained safe in the home at visit completion. Family verbalized understanding of care plan.
Temp 97.7 BP 136/87 HR 61 SPO2 UTO due to cold fingers

Last bowel movement: 12/3/2025
Pain: 2
PRN medication usage last 24 hours: 0
Need medication refills: No
Discussion with caregiver/POA/facility staff: discussed with daughter Pt behavior adverse reactions and new medication
Date: 12/4/2025

Electronically signed by: JOANNIE ESTES RN

Patient: JERILYN HENRY
Medical Record: #97137

NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

Date/Time in:12/11/2025 2:01:00 PM

Date/Time out:12/11/2025 2:02:00 PM

Purpose of progress note:IDG Narrative

IDG Narrative Jerilyn Henry is sitting in a chair beside her bed in her bedroom upon arrival. She was awake and alert and able to answer questions. Although she demonstrated forgetfulness and periods of confusion, she was able to verbalize her needs. Patient was noted to repeat words and questions.

Patient remains ambulatory in the hallways. She is continent of bladder and is able to sit on the commode during the day. Due to cognitive impairment and forgetfulness, patient requires extended time to complete meals, taking up to 45 minutes to finish eating.

She requires maximum assistance with bathing and dressing due to short attention span and is dependent on toileting. Patient appeared calm and comfortable during the visit, with no signs of distress noted.

She has episodes of anxiety and agitation which was reported a week ago. She does not have that this past few days.

Date: 12/11/2025

Electronically signed by: JOCELYN PRUDENTE RN

Patient: JERILYN HENRY
Medical Record: #97137

ADMISSION ORDERS / INITIAL PLAN OF CARE

Skilled Nursing Orders

- ☒ Admit to hospice.
- Level of care: Routine Home Care
- ☒ Assess vital signs and physiological systems as needed.
- ☒ Assess pain and symptoms and need for further intervention.
- ☒ Teach need for giving pain medications around-the-clock and potential side effects of medications.
- ☒ May give medications related to palliative care sublingually or rectally if patient is unable to swallow.
- ☒ May anchor Foley catheter and change/irrigate per hospice protocol.
- ☒ Assess bowel status with each visit.
- ☒ May check for impaction PRN.
- ☒ Oxygen at 2-4 L per nasal cannula PRN for dyspnea and/or respiratory distress.
- ☒ May check pulse oximetry for dyspnea, altered respirations, altered mental status and prior to ordering oxygen. Notify physician if reading is under 90%.
- ☒ Initiate measures to prevent/manage skin breakdown: hydrocolloid dressing, pressure-relieving mattress PRN.
- ☒ Teach and support caregiver in care of patient.
- ☒ Teach safety measures, infection control, and emergency preparedness.
- ☒ Medication orders attached.
- ☒ May place Hospice Emergency Medication Kit in patient's residence and use PRN as directed.

Nutrition Orders

- ☒ Oral diet orders
- Describe: diet as tolerated

Activity Orders

- ☒ As tolerated

Durable Medical Equipment Orders

- ☒ Hospital bed
- ☒ Overbed table
- ☒ Oxygen

INITIAL PLAN OF CARE

Skilled Nursing Visits

- ☒ Supervisory Visits every 2 weeks if patient receives Hospice Aide services
- Weekly Visit Frequency (to be reassessed at next IDG): 2
- +PRN Weekly Visits 1
- Reason for PRN Visits change in condition

Social Worker Visits

- ☒ Psychosocial/Anticipatory Grief/Bereavement assessment
- ☒ Assist patient/family in planning future care
- ☒ Assist with access to community resources
- ☒ Assist with Advance Directives
- ☒ Assist with funeral planning
- ☒ Assist patient/family in obtaining financial assistance
- ☒ Educate patient/family in coping with loss/disease, grief symptoms, anticipatory grief, and available hospice services
- ☒ Evaluation for volunteer services
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Spiritual Visits

- ☒ Spiritual needs assessment
- ☒ Phone contacts and/or visits to patient/family
- ☒ Phone call to patient/family's own pastor
- ☒ Patient/family receive visits from their own pastor
- ☒ Patient/family request no Spiritual Counselor contact at this time
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Hospice Aide Visits

- ☒ Assist with personal care/hygiene and ADLs
- ☒ Light housekeeping
- ☒ Aide Assignment Sheet/Plan of Care completed
- Weekly Visit Frequency (to be reassessed at next IDG): 5

Therapy Visits

- ☒ No therapy needs identified at this time

Volunteer Visits

- ☒ Volunteer services were explained to patient/POA.

Medication Review

- ☒ Medical Director and/or Attending Physician has reviewed all the medications including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.
- ☒ All medications have been reviewed and reconciled with Attending Physician and/or Medical Director.

As the Interdisciplinary Team members, we have participated in the planning of this patient's care prior to providing care (as applicable)

Medical Director/Physician Designee: DIEGO F RESTREPO MD

Clinical Director: TRACY KING RN

Registered Nurse: JOANNIE ESTES RN

Social Worker: ASHLEY DAVIS LMSW

Spiritual Counselor: CLARK MAYS

Volunteer Coordinator: VERNON MORRISON

Bereavement Coordinator: VERNON MORRISON

Date: 11/26/2025

Electronically signed by: TRACY KING RN
Electronically signed by: CLARK MAYS
Electronically signed by: VERNON MORRISON
Electronically signed by: DIEGO F RESTREPO MD

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Diarrhea

Measurable Goals: Patient and family will report understanding of disease process and management of loose, frequent stools.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Diarrhea	2	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Discontinue medications that can contribute to diarrhea: Magnesium-based antacids, Theophylline, Reglan, Antibiotics, NSAIDS Instruct patient to eat small, frequent, relaxed meals.	11/28/2025	TRACY KING RN
2	Potential for Diarrhea	2	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Discontinue medications that can contribute to diarrhea: Magnesium-based antacids, Theophylline, Reglan, Antibiotics, NSAIDS Instruct patient to eat small, frequent, relaxed meals.	12/17/2025 8:47 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Anxiety/Agitation

Measurable Goals: Patient and family/caregiver will report understanding of the causes and treatment of anxiety.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Anxiety/Agitation	2	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Assess level of anxiety and/or depression in caregiver. Review care plan with patient/family/caregiver.	11/28/2025 1:34 PM	TRACY KING RN
2	Potential for Anxiety/Agitation	1	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Assess level of anxiety and/or depression in caregiver. Review care plan with patient/family/caregiver	12/17/2025 8:49 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Cognitive Loss/Dementia

Measurable Goals: Define appropriate support roles for each Nursing Facility and hospice staff member involved in resident's care.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Cognitive Loss/Dementia	1	Assist family in coping with loss of functioning in their loved one. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	11/28/2025 1:36 PM	TRACY KING RN
2	description: Potential for Cognitive Loss/Dementia	1	Assist family in coping with loss of functioning in their loved one. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	12/17/2025 8:48 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Edema/Lymphedema

Measurable Goals: Edema will be reduced or maintained, and complications of edema will be prevented or minimized.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for lymphedema:, Skin care, External compression or support, Manual lymph drainage, Exercise	11/28/2025 1:38 PM	TRACY KING RN
2	Potential for Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for lymphedema:, Skin care, External compression or support, Manual lymph drainage, Exercise	12/17/2025 8:45 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Nausea/Vomiting

Measurable Goals: Family/caregivers will verbalize understanding of the management of nausea and vomiting.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Nausea/Vomiting	2	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to give anti-emetics 30-60 minutes before eating.	11/28/2025 1:40 PM	TRACY KING RN
2	Potential for Nausea/Vomiting	2	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to give anti-emetics 30-60 minutes before eating.	12/17/2025 8:44 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Pain

Measurable Goals: Patient will report that pain is reduced to the level personally desired: _____ on a scale of 1-10

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Pain	2	Ensure breakthrough pain medication is available. Assess pain rating on a scale of 1-10 with every visit. Notify Case Manager if pain is greater than _____.	11/28/2025 1:43 PM	TRACY KING RN
2	Potential for Pain	2	Ensure breakthrough pain medication is available. Assess pain rating on a scale of 1-10 with every visit. Notify Case Manager if pain is greater than _____.	12/17/2025 8:40 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Constipation
Measurable Goals: Patient will report soft, formed stools every one to three days.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Constipation	2	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia Instruct patient and family/caregiver on disease process, bowel management, diet, exercise, fluid intake.	11/28/2025 1:44 PM	TRACY KING RN
2	Potential for Constipation	2	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia Instruct patient and family/caregiver on disease process, bowel management, diet, exercise, fluid intake.	12/17/2025 8:43 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

CARE PLAN

Problem being care planned: Dyspnea
Measurable Goals: Patient will report decreased feelings of breathlessness and increased comfort.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Dyspnea	2	Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites If patient reports improved symptoms with use of oxygen, continue oxygen therapy. Request/administer diuretics and vasodilators for dyspnea caused by CHF.	11/28/2025 1:46 PM	TRACY KING RN
2	Potential for Dyspnea	2	Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites If patient reports improved symptoms with use of oxygen, continue oxygen therapy. Request/administer diuretics and vasodilators for dyspnea caused by CHF.	12/17/2025 8:42 AM	TRACY KING RN

Patient: JERILYN HENRY
Medical Record: #97137

IDG UPDATE

From date: 12/4/2025
To date: 12/18/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 2
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	11/26/2025		24 HR metoprolol succinate 100 MG Extended Release Oral Capsule	100 mg	1 tab Extended Release Oral Capsule	Oral Pill	am	HTN	Yes	Active
	11/26/2025		acetaminophen 500 MG Oral Tablet [Tylenol]	500 mg	1 tab Oral Tablet	po	QID PRN	mild pain	Yes	Active
	11/26/2025		apixaban 2.5 MG Oral Tablet [Eliquis]	2.5 mg	1 tab Oral Tablet	Oral Pill	bid	blood thinner	No	Active
	11/26/2025		aspirin 81 MG	81 mg	1 tab	po	1 tab Daily	analgesics	Yes	Active

		Chewable Tablet		Chewable Tablet					
11/26/2025		donepezil hydrochloride 5 MG Oral Tablet [Aricept]	5 mg	1 tab Oral Tablet	Oral Pill	HS	Behaviors	Yes	Active
11/26/2025		lisinopril 10 MG Oral Tablet	10 mg	1 tab Oral Tablet	Oral Pill	daily	HTN	Yes	Active
11/26/2025		lovastatin 10 MG Oral Tablet	10 mg	1 tab Oral Tablet	Oral Pill	HS	HLD	No	Active
11/26/2025		quetiapine 25 MG Oral Tablet [Seroquel]	25 mg	1 tab Oral Tablet	po	BID	antipsychotic	Yes	Active
11/26/2025		sertraline 100 MG Oral Tablet	100 mg	1 tab Oral Tablet	Oral Pill	am	Depression	Yes	Active
11/26/2025		ubidecarenone 100 MG Oral Capsule [Q-Sorb Co Q-10]	100 mg	1 tab Oral Capsule	Oral Pill	daily AM	HTN	No	Active
12/4/2025		divalproex sodium 125 MG Delayed Release Oral Tablet [Depakote]	125 mg	1 cap Delayed Release Oral Tablet	Oral Pill	BID	behaviors	Yes	Active
Comfort Kit Medications:									
11/26/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
11/26/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
11/26/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
11/26/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
11/26/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
11/26/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
11/26/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
11/26/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 12/4/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Diarrhea	11/28/2025	11/28/2025
Anxiety/Agitation	11/28/2025	11/28/2025
Cognitive Loss/Dementia	11/28/2025	11/28/2025
Edema/Lymphedema	11/28/2025	11/28/2025
Nausea/Vomiting	11/28/2025	11/28/2025
Pain	11/28/2025	11/28/2025
Constipation	11/28/2025	11/28/2025
Dyspnea	11/28/2025	11/28/2025

Care Plan list reflects care plan orders as of 12/4/2025

Treatment Order List:

Name	Start Date	DC Date
No Physician Orders		

Physician orders list reflects physician orders as of 12/4/2025

IDG Narrative: Patient Name: Jerilyn Henry
Age/Sex/Race: 83-year-old Caucasian female
Date of Admission: 11/26/2025

Level of Care: Routine Home Care

Primary Diagnosis: I50.22 - Chronic systolic (congestive) heart failure

Related Comorbidities: G31.1 - Senile degeneration of brain, not elsewhere classified, I48.91 - Unspecified atrial fibrillation, L03.90 - Cellulitis, unspecified, N39.46 - Mixed incontinence, F03.90 - Unsp dementia, unsp severity, without beh/psych/mood/anx, R53.1 - Weakness, Z91.81 - History of falling, R60.9 - Edema, unspecified, R19.7 - Diarrhea, unspecified, R06.03 - Acute respiratory distress
Unrelated Conditions (if applicable): Z85.831 - Personal history of malignant neoplasm of soft tissue
Certifying Physician: Dr. Diego Restrepo

POA/Decision Maker: Step Daughter – Noelle Henry/ David Henry Spouse

Advance Directives: DNR;

Allergies: NKDA, Latex Natural Rubber
Current Scales: PPS: 50% KPS: 50% Mid-Arm Circumference (LMAC): 22
FAST (if dementia): 6E

NYHA Classification: Class IV (SOB at rest)
Subjective Past Medical History / Decline
The patient has a significant cardiac history, including heart surgery with two stents in 2012 and a history of atrial fibrillation, although she was in a regular rhythm at today's visit. She carries a diagnosis of dementia, which has notably worsened over the last two weeks, with increased confusion, withdrawal, reduced ambulation, and poor safety awareness. She is a high fall risk and requires redirection and reminders for most tasks. Patient has a prior history of cellulitis to the leg requiring hospitalization and surgery in 2022 following a fall. She has a history of facial skin cancer and has required treatment in the past. She reports dysuria, periodic diarrhea, and decreased appetite with fatigue and generalized weakness. She reports drinking wine 2–3 times a year. Patients and family verbalize understanding of hospice philosophy and desire comfort-focused care.

Objective
General: Cachectic, Pt SOB at rest SAT 89% on RA she refuses Oxygen. Lungs are clear, diminished throughout.

Neuro: A&O x2 but fatigued and slow to respond. Has a blank stare hoh.

Respiratory: Respirations 17 /min at rest; diminished breath sounds mid to lower lobes throughout; O2 saturation 88–89% on RA, She refuses Oxygen at this time.

Cardiac: HR 71, regular; BP 152/87 Pt with 2 stents placed in 2012, she has Afib

GI: Soft, non-tender; bowel sounds present; reports decreased appetite (50–75% of 2 meals). She may skip breakfast drinking coffee only

GU: Incontinent of urine just started on pull ups, toileting assistance is needed.

Skin: Dry, fragile, no open areas. Scab to left ankle noted d/t previous fall per cg 10 days ago

Extremities: Weak grip, generalized muscle wasting; NO lower-extremity edema.

Mobility: Pt ambulates with FWW high fall risk, weak needs assistance for safety.

Weight Change: unknown last weight per documentation was noted to be 114 lbs a year ago with BMI of 21.56.

Functional Status

Current PPS and KPS are both 50%. She ambulates short distances with a four-wheel walker, but mobility has declined, and she spends increasing time in her room alone. She becomes easily confused, responds “huh?” frequently, and demonstrates poor short-term memory recall. She requires reminders to change clothes and may wear the same clothing for days. She resists showers and prefers “bird baths”; however, she is scheduled for three showers weekly with aide assistance. She is increasingly lethargic and fatigued, stating she “doesn’t sleep,” although observation indicates she naps frequently, approximately 10 hours cumulatively per day. Nutrition / Intake She eats 1–2 meals per day, consuming 50–75% of offered meals. She eats smaller portions at breakfast. She drinks a significant amount of coffee and reports occasional diarrhea. No edema noted.

Respiratory / Cardiovascular During this assessment, oxygen saturation was 88–89% on room air. She is refusing supplemental oxygen at this time. Lung sounds are shallow and clear/diminished throughout. Heart rhythm regular on assessment.

GI / GU Abdomen is soft and non-tender. She is unsure of her last bowel movement but estimates “two days ago.”

Dysuria reported. No bladder distention noted.

Psychosocial / Behavioral Patient is a loner, withdrawn, and prefers to stay to herself. She demonstrates increased anxiety and cognitive decline. She is forgetful and often responds to questions with “huh?” before attempting to answer. She takes quetiapine twice daily for behaviors and mood stabilization. Her husband is currently hospitalized for a hip fracture, which may be contributing to emotional withdrawal and decline.

Spiritual She identifies as Baptist and has Bibles placed throughout her room. She finds comfort in her faith.

Safety High fall risk

Poor safety awareness Poor short-term memory Increased confusion Declining mobility Requires repeated cues and reminders

Assessment Summary The patient appears to be in overall decline, with worsening dementia, increasing weakness, reduced oral intake, increased fatigue, poor hygiene, and decreased ability to perform ADLs independently. Her oxygen saturation remains low on room air, and she refuses supplemental oxygen. Cognitive decline is progressing, and functional status has decreased to PPS/KPS 50%. Family is aware of her decline, agrees with the hospice plan of care, and is supportive at this time. Ongoing education has been provided regarding disease progression, fall risk, and safety needs. Plan Continue routine home care hospice services Monitor cognitive decline, nutrition, hydration, bowel habits Reinforce fall precautions and safety Encourage oxygen use as tolerated Provide aide support for hygiene and showers Provide emotional and spiritual support Continue monitoring for changes in condition, symptoms, or needs Clinical

Summary Statement (CMS Required) Based on clinical findings, performance scale decline, respiratory compromise, progressive weight loss, increased dependence with ADLs, and overall functional decline, the patient demonstrates a terminal trajectory consistent with end-stage COPD. It is the nurse's clinical judgment that the patient has a life expectancy of six months or less should the disease follow its normal course. Hospice care is appropriate and medically necessary.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD

Registered Nurse: JOANNIE ESTES RN

Social Worker: ASHLEY DAVIS LMSW

Spiritual Counselor: CLARK MAYS

Volunteer Coordinator: VERNON MORRISON

Bereavement Coordinator: VERNON MORRISON

Date: 12/4/2025

Electronically signed by: VERNON MORRISON

Electronically signed by: DIEGO F RESTREPO MD

Electronically signed by: VIRGINIA ALLAN RN ADON

Electronically signed by: CLARK MAYS

Patient: JERILYN HENRY
Medical Record: #97137

CHANGE OF FREQUENCY ORDER

Frequencies are being adjusted for the following disciplines:

☒ Hospice Aide

Weekly Visit Frequency: 7
Reason for change: Patient Need

Medical Director/Physician Designee: DIEGO F RESTREPO MD
RN Case Manager: JOANNIE ESTES RN

Date: 12/12/2025

Electronically signed by: TRACY KING RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: JERILYN HENRY
Medical Record: #97137

IDG UPDATE

From date: 12/18/2025
To date: 1/1/2026

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 2
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 7

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	11/26/2025		24 HR metoprolol succinate 100 MG Extended Release Oral Capsule	100 mg	1 tab Extended Release Oral Capsule	Oral Pill	am	HTN	Yes	Active
	11/26/2025		acetaminophen 500 MG Oral Tablet [Tylenol]	500 mg	1 tab Oral Tablet	po	QID PRN	mild pain	Yes	Active
	11/26/2025		apixaban 2.5 MG Oral Tablet [Eliquis]	2.5 mg	1 tab Oral Tablet	Oral Pill	bid	blood thinner	No	Active
	11/26/2025		aspirin 81 MG	81 mg	1 tab	po	1 tab Daily	analgesics	Yes	Active

		Chewable Tablet		Chewable Tablet					
11/26/2025		donepezil hydrochloride 5 MG Oral Tablet [Aricept]	5 mg	1 tab Oral Tablet	Oral Pill	HS	Behaviors	Yes	Active
11/26/2025		lisinopril 10 MG Oral Tablet	10 mg	1 tab Oral Tablet	Oral Pill	daily	HTN	Yes	Active
11/26/2025		lovastatin 10 MG Oral Tablet	10 mg	1 tab Oral Tablet	Oral Pill	HS	HLD	No	Active
11/26/2025		sertraline 100 MG Oral Tablet	100 mg	1 tab Oral Tablet	Oral Pill	am	Depression	Yes	Active
11/26/2025		ubidecarenone 100 MG Oral Capsule [Q-Sorb Co Q-10]	100 mg	1 tab Oral Capsule	Oral Pill	daily AM	HTN	No	Active
12/4/2025		divalproex sodium 125 MG Delayed Release Oral Tablet [Depakote]	125 mg	1 cap Delayed Release Oral Tablet	Oral Pill	BID	behaviors	Yes	Active
Comfort Kit Medications:									
11/26/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
11/26/2025		Atropine sulfate oral Soluntion	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
11/26/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
11/26/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
11/26/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
11/26/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
11/26/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
11/26/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 12/18/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Diarrhea	11/28/2025	12/17/2025
Anxiety/Agitation	11/28/2025	12/17/2025
Cognitive Loss/Dementia	11/28/2025	12/17/2025
Edema/Lymphedema	11/28/2025	12/17/2025
Nausea/Vomiting	11/28/2025	12/17/2025
Pain	11/28/2025	12/17/2025
Constipation	11/28/2025	12/17/2025
Dyspnea	11/28/2025	12/17/2025

Care Plan list reflects care plan orders as of 12/18/2025

Treatment Order List:

Name	Start Date	DC Date
No Physician Orders		

Physician orders list reflects physician orders as of 12/18/2025

IDG Narrative:

Jerilyn Henry is sitting in a chair beside her bed in her bedroom upon arrival. She was awake and alert and able to answer questions. Although she demonstrated forgetfulness and periods of confusion, she was able to verbalize her needs. Patient was noted to repeat words and questions. Patient remains ambulatory in the hallways. She is continent of bladder and is able to sit on the commode during the day. Due to cognitive impairment and forgetfulness, patient requires extended time to complete meals, taking up to 45 minutes to finish eating.

She requires maximum assistance with bathing and dressing due to short attention span and is dependent on toileting. Patient appeared calm and comfortable during the visit, with no signs of distress noted. She has episodes of anxiety and agitation which was reported a week ago. She does not have that this past few days.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient’s care:

- Medical Director/Physician Designee: DIEGO F RESTREPO MD
- Registered Nurse: JOANNIE ESTES RN
- Social Worker: ASHLEY DAVIS LMSW
- Spiritual Counselor: CLARK MAYS
- Volunteer Coordinator: VERNON MORRISON
- Bereavement Coordinator: VERNON MORRISON

Date: 12/18/2025

Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: VERNON MORRISON